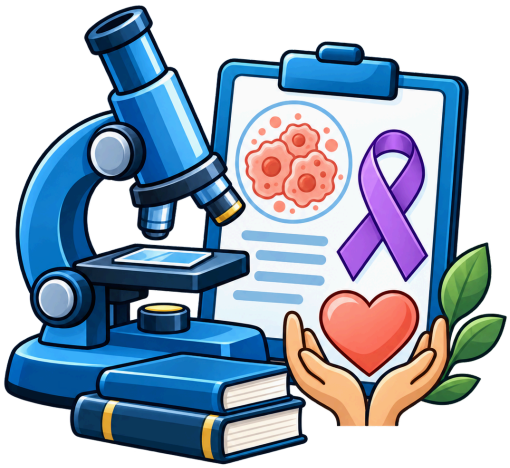
ENGLISH FOR WORK / ENGLISH FOR ONCOLOGISTS

Oncology English

Explain precisely. Leave room to respond.



Phil Stilwell

© 2026 Phil Stilwell. All rights reserved.
No part of this publication may be reproduced, stored in a retrieval system, or transmitted in any form or by any means without prior written permission from the author, except as permitted by law.

Eight practical lessons. Eleven extended conversations.
Vocabulary, phrase banks, guided speaking, and explained answers.

Revised 2026 edition / Upper-intermediate to advanced

Your route through the book

01 Explaining a new cancer diagnosis, page 4

Deliver an established finding in small steps while responding to emotion and information preferences.

02 Separating stage, grade, and prognosis, page 14

Explain closely related oncology terms without converting one into another.

03 Discussing biomarker results, page 26

Explain the difference between a molecular finding, a relevant option, and treatment eligibility.

04 Making treatment intent explicit, page 36

Explain treatment goals and tradeoffs without promising a response or presenting consent as a formality.

05 Escalating a treatment-related symptom report, page 48

Give a concise urgent handoff that distinguishes a reported symptom from an established treatment complication.

06 Discussing a response-assessment scan, page 58

Explain a scan comparison while distinguishing measurement, interpretation, and an agreed treatment decision.

07 Explaining a clinical trial invitation, page 71

Explain research participation while separating eligibility, consent, and proven benefit.

08 Discussing palliative care and priorities, page 81

Explain a supportive-care referral and elicit goals without equating it with abandonment or an automatic change in treatment.

Professional communication notes, page 3

Three additional workplace cases, page 94

Answers and explanations, page 106

Phrase finder, page 160

Vocabulary index, page 162

References and publication notes, page 166

Suggested study range: B2-C1. This is an editorial study range, not a certified proficiency assessment.

Precision without emotional distance.

Oncology conversations often carry unfamiliar terminology and strong emotion at the same time. These original cases practice small information steps, explicit uncertainty, and a clear next conversation.

Ask how much information is wanted

Check the patient's understanding and information preference. Pause after difficult news instead of covering silence with more terminology.
"Would you like the main finding first, then time for questions?"

Separate related technical terms

Stage, grade, response, remission, and cure answer different questions. State what the available evidence supports and what remains unresolved.
"The pathology establishes the diagnosis; staging is still incomplete."

Make the treatment aim explicit

A discussion of benefit needs a stated goal and uncertainty. Avoid describing control, symptom relief, and cure as interchangeable outcomes.
"The aim of this option is disease control; it is not a guarantee."

Connect support with active care

Pain, distress, practical needs, and family communication belong in cancer care. A supportive-care referral does not by itself mean anticancer treatment is ending.
"We can address symptoms alongside the cancer treatment discussion."

Professional boundaries

All people, records, figures, and encounters are fictional. This is professional English practice, not medical advice, clinical training, a treatment protocol, or certification. Use current evidence, authorized supervision where required, local law, and your actual clinical and emergency procedures. Do not use an exercise to decide that symptoms can safely wait, select treatment, or calculate a dose. Communication about privacy, consent, and professional duties depends on jurisdiction and setting. Use qualified language support and protect real patient information.

A Read the situation

LESSON 01 / EXPLAINING A NEW CANCER DIAGNOSIS

Deliver an established finding in small steps while responding to emotion and information preferences.

The first clear sentence

Dr Imani meets Maya after a pathology report confirms a malignant tumor. Staging is incomplete, and no treatment recommendation or prognosis estimate has been established. Maya wants the main finding first and then a pause. Maya has not yet decided whether a relative should join. The clinician must give the finding plainly, acknowledge the response, and separate the diagnosis from unanswered questions.

People: Dr Imani, Oncologist; Maya, Patient.

Read the record

Choose one answer per question using the facts above.

1. What is established?

A. A completed stage
B. A guaranteed treatment outcome
C. A survival estimate
D. A malignant tumor on pathology

2. What does Maya request?

A. The main finding followed by a pause
B. Every statistic immediately
C. Disclosure to all relatives automatically
D. No explanation ever

3. What family decision is unfinished?

A. Whether the relative is the decision-maker
B. Whether privacy no longer applies
C. Whether a relative should join
D. Whether all relatives consented

Answers and explanations, page 106

B Vocabulary in use

CORE TERMS

pathology report
Laboratory interpretation of examined tissue or cells.

review the pathology report

biopsy
Removal of tissue or cells for examination.

explain the biopsy finding

malignancy
Cancerous disease characterized by malignant cells or tissue.

explain a confirmed malignancy

histology
Microscopic study of tissue structure and features.

review the histology

primary tumor
Tumor at the site where the cancer began.

identify the primary tumor

diagnostic disclosure
Communication of an established diagnosis to the patient.

prepare for diagnostic disclosure

warning statement
Brief introduction preparing a listener for difficult information.

use a clear warning statement

information preference
How much detail a person wants and how they want it presented.

check the information preference

emotional acknowledgment
Recognition of the listener's expressed feelings or response.

offer emotional acknowledgment

prognostic uncertainty
Incomplete ability to predict an individual's future disease course.

explain prognostic uncertainty

staging workup
Investigations used to establish the extent of cancer.

complete the staging workup

multidisciplinary team
Professionals from different specialties contributing to a care discussion.

consult the multidisciplinary team

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

pathology report PATHOLOGY REPORT

review the pathology report

Keep the meaning precise: Laboratory interpretation of examined tissue or cells.

diagnostic disclosure DIAGNOSTIC DISCLOSURE

prepare for diagnostic disclosure

Keep the meaning precise: Communication of an established diagnosis to the patient.

B Precision and professional shorthand

EXTEND YOUR RANGE

second opinion

Assessment or advice from another qualified clinician.

arrange a second opinion

support person

Individual the patient wants involved for assistance.

invite a chosen support person

information overload

More information than a listener can usefully process at one time.

avoid information overload

qualified interpreter

Trained language professional supporting accurate communication between speakers.

request a qualified interpreter

patient preference

A person's stated wishes about care or communication.

elicit the patient preference

documented plan

Agreed next steps recorded for the people involved.

review the documented plan

care navigator

Professional helping a person move through services and appointments.

contact the care navigator

diagnostic certainty

Degree to which the diagnosis is established by evidence.

distinguish diagnostic certainty

follow-up consultation

Later meeting to review findings, options, or unresolved questions.

arrange a follow-up consultation

teach-back

Checking an explanation by inviting the listener to restate its meaning.

use teach-back without blame

clinical handoff

Transfer of relevant care information and responsibility between professionals.

confirm the clinical handoff

source attribution

Identification of who reported, observed, or established a fact.

preserve source attribution

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

multidisciplinary team MULTIDISCIPLINARY TEAM

consult the multidisciplinary team

Keep the meaning precise: Professionals from different specialties contributing to a care discussion.

information overload INFORMATION OVERLOAD

avoid information overload

Keep the meaning precise: More information than a listener can usefully process at one time.

C Phrases that move the work forward

Prepare for the finding
I am afraid the result is not what we hoped for.

Name the diagnosis
The tissue examination confirms a malignant tumor.

Allow a pause
We can stop here for a moment.

Acknowledge emotion
I can see that this is a great deal to take in.

Separate questions
We know the diagnosis; we do not yet know the full extent.

Ask about support
Would you like someone you choose to join us?

Limit a prediction
I cannot give an individual outlook from this finding alone.

Offer continuity
We will arrange another conversation when the remaining results are available.

Invite correction
Please stop me if I have misunderstood.

Explain a term
In everyday language, that means the following.

Separate uncertainty
That remains uncertain; it is not a confirmed finding.

Check your explanation
Let me explain that another way.

Make space
We can pause before moving to the next point.

Name responsibility
Let us confirm who will follow up.

Keep a record accurate
I will distinguish what was reported from what was observed.

Confirm an agreement
We have agreed on the next step, not on an outcome.

Precision in real exchanges

Purpose: Acknowledge emotion
Useful wording: I can see that this is a great deal to take in.

Purpose: Separate questions
Useful wording: We know the diagnosis; we do not yet know the full extent.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which opening is clearest?

A. Everything is definitely curable.
B. The stage is already complete.
C. The tissue examination confirms a malignant tumor.
D. Something might be a little unusual, but who knows.

2. Which response respects Maya's preference?

A. Invite relatives without asking.
B. Demand an immediate treatment decision.
C. Pause after the main finding.
D. Continue through every statistic immediately.

3. A colleague needs the term for this meaning: Microscopic study of tissue structure and features. Which term fits?

A. patient preference
B. histology
C. emotional acknowledgment
D. support person

D Choose the response that fits / continued

4. Rebuild the exchange from "The first clear sentence". Dr Imani: "I am afraid the result is not what we hoped for. The tissue examination confirms a malignant tumor." Maya: [missing response] Dr Imani: "Of course. We can pause. I can see this is a great deal to take in." Which response belongs in the gap?

A. I was about to ask how long I have. I hear that there is still prognostic uncertainty.

B. Could someone help me understand the appointments? I am finding even ordinary arrangements difficult to absorb.

C. Please give me a moment. I knew the biopsy might find something, but hearing that is different.

D. When I am ready, could you explain what the pathology report tells you and what it does not?

Answers and explanations, page 106

One useful distinction

The diagnosis is established in this case, but stage, treatment, and prognosis are not. Do not soften a confirmed finding into vagueness or turn it into an unsupported prediction.

E Conversation

The first clear sentence

The first clear sentence

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

information preference / staging workup / second opinion / prognostic uncertainty / information overload / malignant tumor / support person / pathology report / biopsy / care navigator

01 **Dr Imani:** Before we discuss the report, how much detail would you like in this first conversation?

02 **Maya:** The main finding first, please. My [1] _____ may change once I have heard it.

03 **Dr Imani:** I am afraid the result is not what we hoped for. The tissue examination confirms a [2] _____.

04 **Maya:** Please give me a moment. I knew the [3] _____ might find something, but hearing that is different.

05 **Dr Imani:** Of course. We can pause. I can see this is a great deal to take in.

06 **Maya:** When I am ready, could you explain what the [4] _____ tells you and what it does not?

07 **Dr Imani:** It establishes the cancer diagnosis. It does not yet give us every answer about the extent or treatment.

08 **Maya:** So the [5] _____ is a separate part of understanding what happens next?

09 **Dr Imani:** Yes. We need those remaining details before completing the treatment discussion or estimating your individual outlook.

10 **Maya:** I was about to ask how long I have. I hear that there is still [6] _____.

Conversation continues on the next page.

The first clear sentence

CONVERSATION / CONTINUED

11 **Dr Imani:** I will answer as honestly as I can, including what we do not know. I will not invent a number.

12 **Maya:** Thank you. I might want my brother here, but I have not chosen a [7] _____ for this conversation yet.

13 **Dr Imani:** You can decide. We will discuss what you want shared rather than assume that every family member is involved.

14 **Maya:** Could someone help me understand the appointments? I am finding even ordinary arrangements difficult to absorb.

15 **Dr Imani:** Our [8] _____ can help with those arrangements while we keep the clinical questions with the appropriate team.

16 **Maya:** I do not want a large pack of unexplained information to be my only connection with the team.

17 **Dr Imani:** That would risk [9] _____. We will give manageable information and a clear contact route.

18 **Maya:** I may also want another clinician to review the findings once I understand the next steps.

19 **Dr Imani:** We can discuss a [10] _____ and arrange a follow-up conversation. You do not need every decision settled in this first exchange.

20 **Maya:** For now, I understand the diagnosis is confirmed and the full extent is not. Please let us stop there briefly.

CHECK THE GAPS

1. information preference / **2.** malignant tumor / **3.** biopsy / **4.** pathology report / **5.** staging workup / **6.** prognostic uncertainty / **7.** support person / **8.** care navigator / **9.** information overload / **10.** second opinion

Why these answers fit, page 107

F Check accuracy and scope

The first clear sentence

Return to the case

Dr Imani meets Maya after a pathology report confirms a malignant tumor. Staging is incomplete, and no treatment recommendation or prognosis estimate has been established. Maya wants the main finding first and then a pause. Maya has not yet decided whether a relative should join. The clinician must give the finding plainly, acknowledge the response, and separate the diagnosis from unanswered questions.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. Which conclusion exceeds the facts?

A. A later discussion is needed.
B. The diagnosis alone establishes Maya's individual prognosis.
C. Staging remains incomplete.
D. The pathology confirms malignancy.

2. What should precede family involvement?

A. Assuming every relative may receive details
B. Treating attendance as legal authority
C. Posting the result publicly
D. Clarifying Maya's wishes and the appropriate permissions

Answers and explanations, page 108

Listen for the distinction

Swap roles. Repeat the distinction between a confirmed diagnosis and an incomplete stage without adding a prognosis.

A pause is part of the conversation

Silence after difficult news need not be filled with statistics. Check what the person wants next and who they want involved, without assuming either preference.

F Say it / transfer to a new case

Check who should join

Alex wants partner Ren to attend the next results conversation but has not authorized disclosure to other relatives. The next meeting will review outstanding findings; no outcome is supplied.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

outstanding / Ren / relatives / supplied

01 **Alex:** I would like [1] _____ to join.

02 **Clinician:** The permission does not automatically include other [2] _____.

03 **Alex:** Some findings remain [3] _____.

04 **Clinician:** No outcome has been [4] _____.

CHECK THE GAPS

1. Ren / **2.** relatives / **3.** outstanding / **4.** supplied

Why these answers fit, page 109

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "The first clear sentence". Dr Imani: "I will answer as honestly as I can, including what we do not know. I will not invent a number." Maya: [missing response] Dr Imani: "You can decide. We will discuss what you want shared rather than assume that every family member is involved." Which response belongs in the gap?

A. Thank you. I might want my brother here, but I have not chosen a support person for this conversation yet.

B. For now, I understand the diagnosis is confirmed and the full extent is not. Please let us stop there briefly.

C. When I am ready, could you explain what the pathology report tells you and what it does not?

D. I do not want a large pack of unexplained information to be my only connection with the team.

Answers and explanations, page 110

A Read the situation

LESSON 02 / SEPARATING STAGE, GRADE, AND PROGNOSIS

Explain closely related oncology terms without converting one into another.

Two terms on one report

Theo has a pathology report that includes a tumor grade. Imaging needed for staging is still pending. Dr Malik must explain that grade concerns microscopic features while stage describes extent using the applicable cancer-specific system. No grade value, stage, or individual survival estimate is supplied. Theo has mistaken the grade for a completed stage.

People: Theo, Patient; Dr Malik, Oncologist.

Read the record

Choose one answer per question using the facts above.

1. Which item is available?

A. A completed staging conclusion
B. A supplied survival estimate
C. All pending imaging
D. A pathology report containing grade

2. What is pending?

A. Whether Theo attended
B. The word grade on the report
C. A confirmed cure
D. Imaging needed for staging

3. What has Theo confused?

A. Two confirmed diagnoses
B. Grade with stage
C. A treatment cycle with a payment
D. A medicine with an appointment

Answers and explanations, page 111

B Vocabulary in use

CORE TERMS

tumor grade
Classification of microscopic tumor features under the relevant grading system.

explain tumor grade

cancer stage
Description of cancer extent under the applicable staging system.

establish the cancer stage

differentiation
Degree to which tumor cells resemble normal cells of their origin.

describe cellular differentiation

local extent
Spread of a tumor within its original area.

assess local extent

regional lymph node
Lymph node in the region draining or associated with the tumor site.

assess regional lymph nodes

distant metastasis
Cancer spread to a site distant from the primary tumor.

evaluate distant metastasis

clinical stage
Stage based on clinical evaluation before definitive pathological staging when applicable.

record the clinical stage

pathological stage
Stage incorporating surgical and pathological findings where applicable.

distinguish pathological stage

TNM classification
Cancer-specific classification describing tumor, regional nodes, and distant metastasis.

explain the TNM classification

stage group
Overall stage category combining applicable staging information.

confirm the stage group

staging investigation
Test used to help determine cancer extent.

arrange a staging investigation

imaging finding
Observation reported from a diagnostic image.

review an imaging finding

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

tumor grade TUMOR GRADE

explain tumor grade

Keep the meaning precise: Classification of microscopic tumor features under the relevant grading system.

distant metastasis DISTANT METASTASIS

evaluate distant metastasis

Keep the meaning precise: Cancer spread to a site distant from the primary tumor.

B Precision and professional shorthand

EXTEND YOUR RANGE

indeterminate finding

Finding whose nature or significance is not yet established.

explain an indeterminate finding

localized disease

Cancer confined to a defined local site in the relevant context.

describe localized disease

prognostic factor

Feature associated with a disease's expected course or outcome.

interpret a prognostic factor

qualified interpreter

Trained language professional supporting accurate communication between speakers.

request a qualified interpreter

patient preference

A person's stated wishes about care or communication.

elicit the patient preference

documented plan

Agreed next steps recorded for the people involved.

review the documented plan

nodal involvement

Presence of cancer in lymph nodes.

assess nodal involvement

metastatic disease

Cancer that has spread to distant body sites.

discuss metastatic disease

staging completion

Point at which the necessary staging information has been assembled.

confirm staging completion

teach-back

Checking an explanation by inviting the listener to restate its meaning.

use teach-back without blame

clinical handoff

Transfer of relevant care information and responsibility between professionals.

confirm the clinical handoff

source attribution

Identification of who reported, observed, or established a fact.

preserve source attribution

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

imaging finding IMAGING FINDING

review an imaging finding

Keep the meaning precise: Observation reported from a diagnostic image.

prognostic factor PROGNOSTIC FACTOR

interpret a prognostic factor

Keep the meaning precise: Feature associated with a disease's expected course or outcome.

C Phrases that move the work forward

Introduce the contrast

These two terms answer different questions.

Explain stage

Stage describes the extent using the system for this cancer.

Avoid a shortcut

We cannot convert the grade into a stage.

Limit prognosis

A stage label is not an exact prediction for one person.

Theo / in the conversation

I thought that meant you already knew the stage of the cancer.

Theo / in the conversation

Then I may have combined two separate pieces of information when I explained the report at home.

Theo / in the conversation

I also saw a word about how much the cells resemble ordinary tissue.

Theo / in the conversation

What does the staging investigation need to establish beyond what the microscope has shown?

Explain grade

Grade describes microscopic features of the tumor.

Name the missing evidence

The required imaging is still pending.

Preserve uncertainty

Indeterminate does not mean confirmed spread.

Check the contrast

Which term describes extent, and which describes the tissue features?

Dr Malik / in the conversation

Stage answers a different question about the extent of disease.

Dr Malik / in the conversation

The cancer stage is not complete while we are waiting for the required imaging.

Dr Malik / in the conversation

It can inform grading, but it does not by itself tell us where disease is present.

Dr Malik / in the conversation

Depending on the cancer, it examines features such as local extent, nearby nodes, and possible disease elsewhere.

Precision in real exchanges

Purpose: Name the missing evidence
Useful wording: The required imaging is still pending.

Purpose: Avoid a shortcut
Useful wording: We cannot convert the grade into a stage.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which contrast is correct?

A. Stage always means cure.
B. Grade alone proves distant spread.
C. Grade concerns microscopic features; stage concerns extent.
D. Grade and stage are interchangeable.

2. What does pending imaging mean?

A. The required result is not yet available.
B. Spread has been excluded.
C. Spread has been confirmed.
D. Imaging has been canceled permanently.

3. A colleague needs the term for this meaning: Spread of a tumor within its original area. Which term fits?

A. TNM classification
B. localized disease
C. local extent
D. patient preference

D Choose the response that fits / continued

4. Rebuild the exchange from "Two terms on one report". Theo: "I also saw a word about how much the cells resemble ordinary tissue. Where does that fit?" Dr Malik: [missing response] Theo: "What does the staging investigation need to establish beyond what the microscope has shown?" Which response belongs in the gap?

A. Correct. A regional lymph node and a distant site have different roles in the applicable staging system.

B. Depending on the cancer, it examines features such as local extent, nearby nodes, and possible disease elsewhere.

C. Differentiation concerns that resemblance. It can inform grading, but it does not by itself tell us where disease is present.

D. That is understandable. The cancer stage is not complete while we are waiting for the required imaging.

Answers and explanations, page 111

> **One useful distinction**
>
> Grade and stage are not synonyms. Staging systems differ by cancer type; do not assign a TNM category or stage from a vocabulary definition.

E Conversation

Two terms on one report

Two terms on one report

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

Distant metastasis / Tumor grade / Differentiation / indeterminate finding / regional lymph node / local extent / TNM classification / prognostic factor / cancer stage / staging completion

01 **Theo:** My report gives a grade. I thought that meant you already knew the stage of the cancer.

02 **Dr Malik:** [1] _____ describes microscopic features. Stage answers a different question about the extent of disease.

03 **Theo:** Then I may have combined two separate pieces of information when I explained the report at home.

04 **Dr Malik:** That is understandable. The [2] _____ is not complete while we are waiting for the required imaging.

05 **Theo:** I also saw a word about how much the cells resemble ordinary tissue. Where does that fit?

06 **Dr Malik:** [3] _____ concerns that resemblance. It can inform grading, but it does not by itself tell us where disease is present.

07 **Theo:** What does the staging investigation need to establish beyond what the microscope has shown?

08 **Dr Malik:** Depending on the cancer, it examines features such as [4] _____, nearby nodes, and possible disease elsewhere.

09 **Theo:** So a nearby lymph node and a finding in a distant organ would not be described identically?

10 **Dr Malik:** Correct. A [5] _____ and a distant site have different roles in the applicable staging system.

Conversation continues on the next page.

Two terms on one report

CONVERSATION / CONTINUED

11 **Theo:** I have heard people use the word metastatic for any unusual spot. Is that too definite?

12 **Dr Malik:** Yes. An [6] _____ has not yet been characterized. We must not relabel uncertainty as confirmed spread.

13 **Theo:** That distinction helps. I would rather the report's uncertainty stay visible than be translated into the worst possibility.

14 **Dr Malik:** Exactly. [7] _____ is a specific conclusion, not a synonym for every finding that needs further review.

15 **Theo:** Will every cancer use the same letters and numbers once those investigations are complete?

16 **Dr Malik:** No. [8] _____ applies in cancer-specific ways, and some cancers use other systems.

17 **Theo:** And even when the stage is known, it will not tell you exactly what happens to me personally?

18 **Dr Malik:** Correct. A [9] _____ informs a discussion; it is not an exact individual timetable.

19 **Theo:** Then I can say the grade is reported, but the stage still needs the remaining investigation results.

20 **Dr Malik:** Yes. We will confirm [10] _____ and explain the conclusion when the evidence is available.

CHECK THE GAPS

1. Tumor grade / **2.** cancer stage / **3.** Differentiation / **4.** local extent / **5.** regional lymph node / **6.** indeterminate finding / **7.** Distant metastasis / **8.** TNM classification / **9.** prognostic factor / **10.** staging completion

Why these answers fit, page 112

F Check accuracy and scope

Two terms on one report

Return to the case

Theo has a pathology report that includes a tumor grade. Imaging needed for staging is still pending. Dr Malik must explain that grade concerns microscopic features while stage describes extent using the applicable cancer-specific system. No grade value, stage, or individual survival estimate is supplied. Theo has mistaken the grade for a completed stage.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. Which wording preserves an indeterminate finding?

A. It must be deleted.
B. Its significance is not yet established.
C. It is definitely metastatic.
D. It is definitely harmless.

2. Which claim is unsupported?

A. The terms need explaining.
B. The grade supplies Theo's exact survival time.
C. The report contains grade.
D. Staging remains incomplete.

Answers and explanations, page 114

Listen for the distinction

Swap roles. Repeat the final summary without supplying a stage or prognosis that the case does not contain.

Correct the interpretation, not the person

Technical terms can appear together on reports while answering different questions. Use a clear contrast and a brief check-back rather than telling the patient they read it incorrectly.

F Say it / transfer to a new case

Keep an uncertain scan finding uncertain

A report calls a lung finding indeterminate. The oncology team is reviewing it. No metastatic diagnosis or stage change has been established.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

indeterminate / established / confirmed / reviewing

01 **Patient:** The report calls the finding [1] _____.

02 **Clinician:** The team is still [2] _____ it.

03 **Patient:** Spread has not been [3] _____.

04 **Clinician:** No stage change is [4] _____.

CHECK THE GAPS

1. indeterminate / **2.** reviewing / **3.** established / **4.** confirmed

Why these answers fit, page 114

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "Two terms on one report". Theo: "That distinction helps. I would rather the report's uncertainty stay visible than be translated into the worst possibility." Dr Malik: [missing response] Theo: "Will every cancer use the same letters and numbers once those investigations are complete?" Which response belongs in the gap?

A. That is understandable. The cancer stage is not complete while we are waiting for the required imaging.

B. Correct. A regional lymph node and a distant site have different roles in the applicable staging system.

C. Exactly. Distant metastasis is a specific conclusion, not a synonym for every finding that needs further review.

D. Depending on the cancer, it examines features such as local extent, nearby nodes, and possible disease elsewhere.

Answers and explanations, page 115

Review 1 / Meaning and context

Use language from the lessons you have completed. Choose one answer per question.

> **Case file**
>
> Revisit "Explaining a new cancer diagnosis" and "Separating stage, grade, and prognosis". Choose precise terminology, then reconstruct the missing responses using both neighboring turns. The final two questions also revisit earlier conversations.

1. A colleague needs the term for this meaning: Cancerous disease characterized by malignant cells or tissue. Which term fits?

A. care navigator
B. teach-back
C. malignancy
D. information preference

2. A colleague needs the term for this meaning: Stage incorporating surgical and pathological findings where applicable. Which term fits?

A. pathological stage
B. indeterminate finding
C. tumor grade
D. qualified interpreter

3. Rebuild the exchange from "The first clear sentence". Dr Imani: "It establishes the cancer diagnosis. It does not yet give us every answer about the extent or treatment." Maya: [missing response] Dr Imani: "Yes. We need those remaining details before completing the treatment discussion or estimating your individual outlook." Which response belongs in the gap?

A. The main finding first, please. My information preference may change once I have heard it.
B. So the staging workup is a separate part of understanding what happens next?
C. Could someone help me understand the appointments? I am finding even ordinary arrangements difficult to absorb.
D. I may also want another clinician to review the findings once I understand the next steps.

Review 1 / Meaning and context / continued

4. Rebuild the exchange from "Two terms on one report". Theo: "And even when the stage is known, it will not tell you exactly what happens to me personally?" Dr Malik: [missing response] Theo: "Then I can say the grade is reported, but the stage still needs the remaining investigation results." Which response belongs in the gap?

A. Yes. We will confirm staging completion and explain the conclusion when the evidence is available.

B. Correct. A prognostic factor informs a discussion; it is not an exact individual timetable.

C. No. TNM classification applies in cancer-specific ways, and some cancers use other systems.

D. Tumor grade describes microscopic features. Stage answers a different question about the extent of disease.

Answers and explanations, page 146

A Read the situation

LESSON 03 / DISCUSSING BIOMARKER RESULTS

Explain the difference between a molecular finding, a relevant option, and treatment eligibility.

> **A possible match is not a promise**
>
> A tumor biomarker report identifies a potentially relevant molecular alteration for Quinn. Dr Sato has not established that a particular treatment is appropriate or available. Clinical context, the strength of evidence, and access need review. The report is tumor testing; no inherited finding has been established. Quinn hopes the result guarantees an effective targeted treatment.

People: Dr Sato, Oncologist; Quinn, Patient.

Read the record

Choose one answer per question using the facts above.

1. What does the report identify?

A. Guaranteed treatment benefit
B. Confirmed inherited risk
C. An approved appointment date
D. A potentially relevant tumor alteration

2. What is not established?

A. Suitability or availability of a particular treatment
B. That a report exists
C. That Quinn has questions
D. That clinical review is needed

3. What kind of testing is described?

A. Tumor testing
B. Confirmed germline testing
C. Testing every relative
D. A completed trial enrollment

Answers and explanations, page 116

B Vocabulary in use

CORE TERMS

biomarker
Measurable feature providing information about a biological process or disease.

interpret a tumor biomarker

targeted therapy
Treatment directed at a relevant molecular target involved in disease.

evaluate a targeted therapy

germline variant
Genetic change in the inherited genetic material that may be passed to offspring.

assess a germline variant

actionability
Degree to which a finding can usefully inform a clinical action.

assess clinical actionability

companion diagnostic
Test providing information essential for appropriate use of a corresponding treatment.

verify a companion diagnostic

sequencing panel
Test examining a selected set of genetic targets.

interpret a sequencing panel

molecular alteration
Change in a molecular feature such as DNA sequence or gene activity.

characterize a molecular alteration

somatic variant
Genetic change acquired in body cells rather than inherited through germ cells.

explain a somatic variant

variant of uncertain significance
Genetic change whose clinical meaning is not established.

report a variant of uncertain significance

evidence level
Strength or category of support for a clinical claim or action.

review the evidence level

tumor profiling
Characterization of molecular features of a tumor.

review tumor profiling

specimen adequacy
Whether a sample meets the requirements for a reliable analysis.

confirm specimen adequacy

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

biomarker BIOMARKER

interpret a tumor biomarker

Keep the meaning precise: Measurable feature providing information about a biological process or disease.

variant of uncertain significance VARIANT OF UNCERTAIN SIGNIFICANCE

report a variant of uncertain significance

Keep the meaning precise: Genetic change whose clinical meaning is not established.

B Precision and professional shorthand

EXTEND YOUR RANGE

tumor heterogeneity

Differences among cells or regions within or between tumors.

discuss tumor heterogeneity

acquired resistance

Reduced treatment sensitivity developing during or after exposure.

investigate acquired resistance

predictive biomarker

Feature associated with likelihood of response to a particular treatment.

interpret a predictive biomarker

genetic counseling

Professional support in understanding genetic findings and their implications.

arrange genetic counseling

testing limitation

Restriction on what a test can detect or establish.

explain a testing limitation

treatment access

Practical ability to obtain an appropriate treatment.

clarify treatment access

qualified interpreter

Trained language professional supporting accurate communication between speakers.

request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

use teach-back without blame

patient preference

A person's stated wishes about care or communication.

elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

preserve source attribution

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

specimen adequacy SPECIMEN ADEQUACY

confirm specimen adequacy

Keep the meaning precise: Whether a sample meets the requirements for a reliable analysis.

testing limitation TESTING LIMITATION

explain a testing limitation

Keep the meaning precise: Restriction on what a test can detect or establish.

C Phrases that move the work forward

Acknowledge hope

I understand why a possible match feels important.

Separate two steps

Finding a target and choosing a treatment are different steps.

Explain review

We need to assess the evidence and your clinical circumstances.

Name access

We also need to establish whether an appropriate option is available.

Dr Sato / in the conversation

What have you understood from it so far, and what would you like clarified?

Dr Sato / in the conversation

The finding may be relevant, but it is the start of another clinical discussion.

Dr Sato / in the conversation

We need to review the supporting evidence and whether the finding fits an appropriate option for you.

Quinn / in the conversation

A long report can look more certain than the information really is.

State the finding

The report identifies a tumor alteration that may be relevant.

Limit a promise

This does not guarantee that a treatment will work.

Separate inheritance

Tumor testing does not automatically establish an inherited change.

Protect uncertainty

Uncertain significance is not the same as proven actionability.

Quinn / in the conversation

I hoped that meant there was a medicine certain to work.

Quinn / in the conversation

Is this the difference between a finding and its actionability, rather than just a complicated way to say the same thing?

Quinn / in the conversation

I would like you to explain the evidence level.

Dr Sato / in the conversation

A possible target does not guarantee a response to the associated treatment.

Precision in real exchanges

Purpose: Limit a promise
Useful wording: This does not guarantee that a treatment will work.

Purpose: Explain review
Useful wording: We need to assess the evidence and your clinical circumstances.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which statement preserves the result accurately?

A. The alteration may be relevant; suitability still needs review.
B. A targeted treatment is guaranteed to work.
C. Every reported variant is actionable.
D. Testing proves every relative is affected.

2. Which distinction matters for relatives?

A. Tumor findings are not automatically inherited findings.
B. Every somatic variant is inherited.
C. All tumor tests test the entire family.
D. Germline and somatic mean identical things.

3. A colleague needs the term for this meaning: Genetic change acquired in body cells rather than inherited through germ cells. Which term fits?

A. patient preference
B. predictive biomarker
C. somatic variant
D. companion diagnostic

D Choose the response that fits / continued

4. Rebuild the exchange from "A possible match is not a promise". Dr Sato: "Yes. We need to review the supporting evidence and whether the finding fits an appropriate option for you." Quinn: [missing response] Dr Sato: "That is a fair request. A possible target does not guarantee a response to the associated treatment." Which response belongs in the gap?

A. I would like you to explain the evidence level. A long report can look more certain than the information really is.

B. Is this the difference between a finding and its actionability, rather than just a complicated way to say the same thing?

C. I also noticed a different entry marked variant of uncertain significance. I should not treat that as another proven treatment target.

D. A testing limitation is important to me because I do not want a negative entry interpreted as an answer to every question.

Answers and explanations, page 116

> **One useful distinction**
>
> A molecular alteration does not guarantee a useful treatment match or response. A tumor finding is not automatically an inherited finding.

E Conversation

A possible match is not a promise

A possible match is not a promise

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

molecular alteration / somatic variant / genetic counseling / targeted therapy / actionability / testing limitation / variant of uncertain significance / treatment access / germline variant / evidence level

01 **Dr Sato:** The tumor report is available. What have you understood from it so far, and what would you like clarified?

02 **Quinn:** It lists a [1] _____. I hoped that meant there was a medicine certain to work.

03 **Dr Sato:** I understand that hope. The finding may be relevant, but it is the start of another clinical discussion.

04 **Quinn:** Is this the difference between a finding and its [2] _____, rather than just a complicated way to say the same thing?

05 **Dr Sato:** Yes. We need to review the supporting evidence and whether the finding fits an appropriate option for you.

06 **Quinn:** I would like you to explain the [3] _____. A long report can look more certain than the information really is.

07 **Dr Sato:** That is a fair request. A possible target does not guarantee a response to the associated treatment.

08 **Quinn:** Then [4] _____ describes how an option works, not a promise of benefit for me.

09 **Dr Sato:** Correct. The clinical setting and other factors matter, and we also need to establish availability.

10 **Quinn:** Please include [5] _____ in that discussion. I cannot assume something listed in a report is obtainable here.

Conversation continues on the next page.

A possible match is not a promise

CONVERSATION / CONTINUED

11 **Dr Sato:** We will. You also asked whether this result means your children have the same genetic change.

12 **Quinn:** Yes. I was unsure whether a [6] _____ was the same as something inherited through the family.

13 **Dr Sato:** It is not the same category. This is tumor testing; it does not automatically establish an inherited finding.

14 **Quinn:** So a [7] _____ would require the appropriate separate interpretation and, where indicated, further assessment.

15 **Dr Sato:** Exactly. We can discuss a genetics referral if appropriate, without declaring an inherited result that we do not have.

16 **Quinn:** I also noticed a different entry marked [8] _____. I should not treat that as another proven treatment target.

17 **Dr Sato:** Correct. We will explain the limits of the test as well as the findings it reports.

18 **Quinn:** A [9] _____ is important to me because I do not want a negative entry interpreted as an answer to every question.

19 **Dr Sato:** We will review those limits, the possible options, and the next steps together before making recommendations.

20 **Quinn:** And [10] _____ can help if the inherited-risk question needs specialist discussion, rather than me guessing from the report.

CHECK THE GAPS

1. molecular alteration / **2.** actionability / **3.** evidence level / **4.** targeted therapy / **5.** treatment access / **6.** somatic variant / **7.** germline variant / **8.** variant of uncertain significance / **9.** testing limitation / **10.** genetic counseling

Why these answers fit, page 117

F Check accuracy and scope

A possible match is not a promise

Return to the case

A tumor biomarker report identifies a potentially relevant molecular alteration for Quinn. Dr Sato has not established that a particular treatment is appropriate or available. Clinical context, the strength of evidence, and access need review. The report is tumor testing; no inherited finding has been established. Quinn hopes the result guarantees an effective targeted treatment.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. What does uncertain significance mean?

A. The report is always wrong.
B. The condition is certainly inherited.
C. Clinical meaning is not established.
D. A treatment must be started immediately.

2. Which additional issue belongs in the discussion?

A. Automatic trial enrollment
B. Evidence, clinical context, and treatment access
C. Only the color of the report
D. A guaranteed response date

Answers and explanations, page 119

Listen for the distinction

Swap roles. Repeat the somatic and germline distinction without declaring an inherited result.

Hope does not require a guarantee

Acknowledge why a possible treatment match matters. Explain the remaining checks without presenting them as bureaucratic obstacles or erasing the potential relevance.

F Say it / transfer to a new case

A result with insufficient sample

A profiling report says the sample was insufficient for one analysis. The result for that target is unavailable, not negative. A clinician will discuss whether further testing is appropriate.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

review / negative / insufficient / unavailable

01 **Patient:** The sample was [1] _____ for that analysis.

02 **Clinician:** The target result is [2] _____.

03 **Patient:** That is not the same as a [3] _____ result.

04 **Clinician:** Further testing needs clinical [4] _____.

CHECK THE GAPS

1. insufficient / **2.** unavailable / **3.** negative / **4.** review

Why these answers fit, page 119

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "A possible match is not a promise". Dr Sato: "It is not the same category. This is tumor testing; it does not automatically establish an inherited finding." Quinn: [missing response] Dr Sato: "Exactly. We can discuss a genetics referral if appropriate, without declaring an inherited result that we do not have." Which response belongs in the gap?

A. Then targeted therapy describes how an option works, not a promise of benefit for me.

B. Yes. I was unsure whether a somatic variant was the same as something inherited through the family.

C. So a germline variant would require the appropriate separate interpretation and, where indicated, further assessment.

D. It lists a molecular alteration. I hoped that meant there was a medicine certain to work.

Answers and explanations, page 120

A Read the situation

LESSON 04 / MAKING TREATMENT INTENT EXPLICIT

Explain treatment goals and tradeoffs without promising a response or presenting consent as a formality.

> **What is this option trying to achieve?**
>
> Dr Bell discusses a proposed treatment option with Jordan. In this fictional case its stated aim is disease control, not a promised cure. The benefits, burdens, alternatives, and practical effects still need an individual discussion. Jordan prioritizes being able to care for a partner. No regimen, dose, response probability, or final decision is supplied.

People: Jordan, Patient; Dr Bell, Oncologist.

Read the record

Choose one answer per question using the facts above.

1. What is the proposed aim?

A. A completed response
B. A fixed survival extension
C. Disease control
D. A guaranteed cure

2. What matters particularly to Jordan?

A. Avoiding every future discussion
B. Caring for a partner
C. A decision already made
D. A specific dose supplied here

3. What remains undecided?

A. The final treatment choice
B. Whether Jordan has a partner
C. Whether a discussion is taking place
D. Whether the option has an aim

Answers and explanations, page 121

B Vocabulary in use

CORE TERMS

treatment intent
Goal a treatment is intended to pursue.

clarify treatment intent

disease control
Limiting cancer growth or spread as a treatment goal.

discuss disease control

adjuvant treatment
Additional treatment after a primary treatment to reduce recurrence risk when appropriate.

discuss adjuvant treatment

local treatment
Treatment directed at a particular body area.

discuss local treatment

treatment cycle
Repeated unit of a planned treatment schedule.

explain a treatment cycle

quality of life
Person's experienced well-being across relevant areas of life.

discuss quality of life

curative intent
Treatment aim of eliminating the cancer.

explain curative intent

neoadjuvant treatment
Treatment given before the main local treatment in an applicable plan.

explain neoadjuvant treatment

systemic therapy
Treatment acting throughout the body rather than only at one local site.

explain systemic therapy

treatment regimen
Planned combination and schedule of treatment components.

review the treatment regimen

performance status
Clinical description of a person's activity and functional ability.

assess performance status

treatment burden
Physical, practical, and emotional demands associated with treatment.

assess treatment burden

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

treatment intent TREATMENT INTENT

clarify treatment intent

Keep the meaning precise: Goal a treatment is intended to pursue.

systemic therapy SYSTEMIC THERAPY

explain systemic therapy

Keep the meaning precise: Treatment acting throughout the body rather than only at one local site.

B Precision and professional shorthand

EXTEND YOUR RANGE

benefit-risk balance

Weighing expected benefits against possible harms.

explain the benefit-risk balance

alternative option

Another reasonable approach available for discussion.

compare alternative options

supportive treatment

Intervention addressing symptoms or treatment-related difficulties.

coordinate supportive treatment

informed consent

Voluntary agreement after an appropriate explanation and opportunity for questions.

support informed consent

decision capacity

Ability to make the relevant decision under the applicable assessment.

assess decision capacity

caregiving responsibility

Duties involved in helping another person with care.

discuss caregiving responsibilities

qualified interpreter

Trained language professional supporting accurate communication between speakers.

request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

use teach-back without blame

patient preference

A person's stated wishes about care or communication.

elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

preserve source attribution

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

treatment burden TREATMENT BURDEN

assess treatment burden

Keep the meaning precise: Physical, practical, and emotional demands associated with treatment.

decision capacity DECISION CAPACITY

assess decision capacity

Keep the meaning precise: Ability to make the relevant decision under the applicable assessment.

C Phrases that move the work forward

State the aim

The aim of this option is to control the disease.

Ask about daily life

How would the treatment demands affect caring for your partner?

Invite alternatives

Let us compare the reasonable alternatives for your circumstances.

Check the goal

What do you understand this option is trying to achieve?

Jordan / in the conversation

Before we go through appointments, I need to understand what you hope this treatment will do.

Dr Bell / in the conversation

I am not describing it as a guaranteed cure.

Dr Bell / in the conversation

Disease control means trying to limit growth or spread; the possible benefit and uncertainty need an individual discussion.

Dr Bell / in the conversation

Your caregiving responsibility belongs in this conversation.

Avoid a guarantee

An intended benefit is not a promised outcome.

Explain the burden

We should discuss visits, symptoms, and practical demands together.

Keep choice open

You have not made a final decision in this discussion.

Respect questions

Questions are part of the consent discussion, not an obstacle to it.

Dr Bell / in the conversation

The treatment intent here is disease control.

Jordan / in the conversation

That is clearer than saying it will help.

Jordan / in the conversation

Repeated hospital visits could affect both of us, not only me.

Dr Bell / in the conversation

Let us examine those demands alongside the clinical aims.

Precision in real exchanges

Purpose: Explain the burden
Useful wording: We should discuss visits, symptoms, and practical demands together.

Purpose: Invite alternatives
Useful wording: Let us compare the reasonable alternatives for your circumstances.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which statement accurately names the goal?

A. This option guarantees cure.
B. A response has already occurred.
C. The outcome is certain.
D. This option aims at disease control.

2. Which question elicits a relevant priority?

A. How would the treatment demands affect caring for your partner?
B. Why does your partner matter?
C. Can we ignore your home responsibilities?
D. You agree that practical issues are irrelevant?

3. A colleague needs the term for this meaning: Treatment given before the main local treatment in an applicable plan. Which term fits?

A. neoadjuvant treatment
B. patient preference
C. treatment cycle
D. supportive treatment

D Choose the response that fits / continued

4. Rebuild the exchange from "What is this option trying to achieve?". Jordan: "I care for my partner. Repeated hospital visits could affect both of us, not only me." Dr Bell: [missing response] Jordan: "I do not want practical difficulties to sound as though I am refusing treatment without listening." Which response belongs in the gap?

A. Your quality of life is important. We need to understand the outcomes that matter to you.

B. We should compare each alternative option, including the expected benefits, limitations, and demands relevant to you.

C. The treatment intent here is disease control. I am not describing it as a guaranteed cure.

D. Your caregiving responsibility belongs in this conversation. Let us examine those demands alongside the clinical aims.

Answers and explanations, page 121

> **One useful distinction**
>
> Curative intent states an aim, not a guarantee. Disease control and symptom relief are also meaningful but different goals.

E Conversation

What is this option trying to achieve?

What is this option trying to achieve?

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

Treatment burden / quality of life / alternative option / caregiving responsibility / benefit-risk balance / treatment cycle / Disease control / Informed consent / treatment regimen / treatment intent

01 **Jordan:** Before we go through appointments, I need to understand what you hope this treatment will do.

02 **Dr Bell:** The [1] _____ here is disease control. I am not describing it as a guaranteed cure.

03 **Jordan:** That is clearer than saying it will help. Does control mean stopping everything permanently?

04 **Dr Bell:** [2] _____ means trying to limit growth or spread; the possible benefit and uncertainty need an individual discussion.

05 **Jordan:** I care for my partner. Repeated hospital visits could affect both of us, not only me.

06 **Dr Bell:** Your [3] _____ belongs in this conversation. Let us examine those demands alongside the clinical aims.

07 **Jordan:** I do not want practical difficulties to sound as though I am refusing treatment without listening.

08 **Dr Bell:** They do not. [4] _____ includes visits and disruption as well as physical effects.

09 **Jordan:** Could you explain the proposed schedule, then distinguish what is definite from what may change?

10 **Dr Bell:** We would review the [5] _____ and the circumstances in which it might need review.

Conversation continues on the next page.

What is this option trying to achieve?

CONVERSATION / CONTINUED

11 **Jordan:** I have heard cycle used as though everyone knows what it means. Please explain that too.

12 **Dr Bell:** A [6] _____ is a repeated unit of the schedule. Its details depend on the actual plan.

13 **Jordan:** I would also like to understand the other reasonable options, not just the one named first.

14 **Dr Bell:** We should compare each [7] _____, including the expected benefits, limitations, and demands relevant to you.

15 **Jordan:** Being able to function at home matters to me. A scan is not the only thing I will judge by.

16 **Dr Bell:** Your [8] _____ is important. We need to understand the outcomes that matter to you.

17 **Jordan:** I am not ready to sign anything just because an appointment slot might be available.

18 **Dr Bell:** [9] _____ is a discussion and a voluntary decision, not simply obtaining a signature.

19 **Jordan:** Then let us compare the possible benefit with the burdens before calling this a settled plan.

20 **Dr Bell:** Yes. We will review the [10] _____ and your priorities; no final choice has been made in this conversation.

CHECK THE GAPS

1. treatment intent / **2.** Disease control / **3.** caregiving responsibility / **4.** Treatment burden / **5.** treatment regimen / **6.** treatment cycle / **7.** alternative option / **8.** quality of life / **9.** Informed consent / **10.** benefit-risk balance

Why these answers fit, page 122

F Check accuracy and scope

What is this option trying to achieve?

> **Return to the case**
>
> Dr Bell discusses a proposed treatment option with Jordan. In this fictional case its stated aim is disease control, not a promised cure. The benefits, burdens, alternatives, and practical effects still need an individual discussion. Jordan prioritizes being able to care for a partner. No regimen, dose, response probability, or final decision is supplied.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. Which account of consent is incomplete?

A. Questions need attention.
B. Alternatives need discussion.
C. The decision must be voluntary.
D. A signature alone proves understanding.

2. Which distinction is correct?

A. Curative intent is an aim, not a guarantee.
B. Disease control means certain cure.
C. Treatment burden means only cost.
D. Quality of life means only a scan result.

Answers and explanations, page 124

> **Listen for the distinction**
>
> Swap roles. Repeat the caregiving-priority question and the consent explanation without choosing a treatment for Jordan.

> **Ask what the goal means in daily life**
>
> A treatment goal has both a clinical meaning and a personal significance. A patient may value time at home, symptom relief, or a particular activity differently from the clinician's initial assumption.

F Say it / transfer to a new case

A goal before a schedule

Reese asks about a proposed option whose stated aim is symptom relief. The schedule has not been agreed. Reese wants its practical demands explained before deciding.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

demands / agreed / open / relief

01 **Clinician:** The stated aim is symptom [1] _____.

02 **Reese:** The schedule is not yet [2] _____.

03 **Clinician:** We need to explain the practical [3] _____.

04 **Reese:** My decision remains [4] _____.

CHECK THE GAPS

1. relief / **2.** agreed / **3.** demands / **4.** open

Why these answers fit, page 124

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "What is this option trying to achieve?". Jordan: "I would also like to understand the other reasonable options, not just the one named first." Dr Bell: [missing response] Jordan: "Being able to function at home matters to me. A scan is not the only thing I will judge by." Which response belongs in the gap?

A. We should compare each alternative option, including the expected benefits, limitations, and demands relevant to you.

B. The treatment intent here is disease control. I am not describing it as a guaranteed cure.

C. Your caregiving responsibility belongs in this conversation. Let us examine those demands alongside the clinical aims.

D. Your quality of life is important. We need to understand the outcomes that matter to you.

Answers and explanations, page 125

Review 2 / Meaning and context

Use language from the lessons you have completed. Choose one answer per question.

> **Case file**
>
> Revisit "Discussing biomarker results" and "Making treatment intent explicit". Choose precise terminology, then reconstruct the missing responses using both neighboring turns. The final two questions also revisit earlier conversations.

1. A colleague needs the term for this meaning: Treatment directed at a relevant molecular target involved in disease. Which term fits?

A. evidence level
B. acquired resistance
C. targeted therapy
D. teach-back

2. A colleague needs the term for this meaning: Planned combination and schedule of treatment components. Which term fits?

A. treatment intent
B. qualified interpreter
C. treatment regimen
D. benefit-risk balance

Review 2 / Meaning and context / continued

3. Rebuild the exchange from "Two terms on one report". Theo: "So a nearby lymph node and a finding in a distant organ would not be described identically?" Dr Malik: [missing response] Theo: "I have heard people use the word metastatic for any unusual spot. Is that too definite?" Which response belongs in the gap?

A. Correct. A regional lymph node and a distant site have different roles in the applicable staging system.

B. That is understandable. The cancer stage is not complete while we are waiting for the required imaging.

C. Depending on the cancer, it examines features such as local extent, nearby nodes, and possible disease elsewhere.

D. Yes. An indeterminate finding has not yet been characterized. We must not relabel uncertainty as confirmed spread.

4. Rebuild the exchange from "What is this option trying to achieve?". Jordan: "I am not ready to sign anything just because an appointment slot might be available." Dr Bell: [missing response] Jordan: "Then let us compare the possible benefit with the burdens before calling this a settled plan." Which response belongs in the gap?

A. We would review the treatment regimen and the circumstances in which it might need review.

B. They do not. Treatment burden includes visits and disruption as well as physical effects.

C. The treatment intent here is disease control. I am not describing it as a guaranteed cure.

D. Informed consent is a discussion and a voluntary decision, not simply obtaining a signature.

Answers and explanations, page 148

A Read the situation

LESSON 05 / ESCALATING A TREATMENT-RELATED SYMPTOM REPORT

Give a concise urgent handoff that distinguishes a reported symptom from an established treatment complication.

> **The call cannot become a routine message**
>
> Oncology nurse Ana calls Dr Ford about Lee, who reports fever and shaking after recent systemic treatment. The urgent oncology assessment pathway has already been activated according to the service protocol. The cause and laboratory results are unknown. The clinicians must exchange the relevant history and confirm responsibility; this case provides no triage threshold, diagnosis, or dosing instruction.

People: Ana, Oncology nurse; Dr Ford, Oncologist.

Read the record

Choose one answer per question using the facts above.

1. What is reported?

A. A completed response assessment
B. No symptoms
C. Fever and shaking after recent treatment
D. A confirmed laboratory diagnosis

2. What action is already taken?

A. All results are normal
B. The urgent assessment pathway is activated
C. A routine message is awaiting next week
D. A dose has been selected here

3. What remains unknown?

A. Whether symptoms were reported
B. Whether treatment was recent
C. Cause and laboratory results
D. Who called Dr Ford

Answers and explanations, page 126

B Vocabulary in use

CORE TERMS

treatment-related toxicity
Harmful effect attributed to treatment after appropriate assessment.

assess suspected treatment-related toxicity

febrile episode
Period during which fever occurs.

report a febrile episode

neutropenia
Abnormally low neutrophil count.

assess for neutropenia

neutrophil count
Measurement of the number of neutrophils in blood.

review the neutrophil count

immunosuppression
Reduced activity or effectiveness of immune defenses.

consider immunosuppression

infection concern
Suspicion that infection may be relevant to a clinical presentation.

escalate an infection concern

infusion reaction
Adverse response occurring in association with an infusion.

report a suspected infusion reaction

extravasation
Escape of an infused substance from a vessel into surrounding tissue.

escalate suspected extravasation

mucositis
Inflammation affecting mucous membranes.

describe mucositis symptoms

dehydration
Deficit of body water requiring clinical assessment in context.

assess suspected dehydration

adverse event
Unfavorable occurrence during care, not necessarily caused by treatment.

report an adverse event

causality assessment
Evaluation of whether one event caused another.

preserve uncertainty in causality assessment

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

treatment-related toxicity TREATMENT-RELATED TOXICITY

assess suspected treatment-related toxicity

Keep the meaning precise: Harmful effect attributed to treatment after appropriate assessment.

infection concern INFECTION CONCERN

escalate an infection concern

Keep the meaning precise: Suspicion that infection may be relevant to a clinical presentation.

B Precision and professional shorthand

EXTEND YOUR RANGE

symptom onset time

Time when the reported symptom began.

confirm symptom onset time

last treatment date

Most recent date on which treatment was administered.

verify the last treatment date

urgent assessment

Prompt clinical evaluation through the appropriate service pathway.

arrange urgent assessment

receiving clinician

Professional taking responsibility for the next clinical assessment.

identify the receiving clinician

read-back

Repetition of important information to verify its accuracy.

use read-back for key details

escalation acknowledgment

Confirmation that an urgent request has been received and accepted.

obtain escalation acknowledgment

qualified interpreter

Trained language professional supporting accurate communication between speakers.

request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

use teach-back without blame

patient preference

A person's stated wishes about care or communication.

elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

preserve source attribution

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

causality assessment CAUSALITY ASSESSMENT

preserve uncertainty in causality assessment

Keep the meaning precise: Evaluation of whether one event caused another.

read-back READ-BACK

use read-back for key details

Keep the meaning precise: Repetition of important information to verify its accuracy.

C Phrases that move the work forward

Lead with urgency

I am calling about an urgent symptom report after recent treatment.

Attribute the report

Lee reports fever and shaking.

Avoid premature diagnosis

I am reporting the concern, not a confirmed cause.

Read back

I will repeat the key details to check accuracy.

State the action

The urgent assessment pathway is already activated.

Preserve an unknown

We do not yet have the laboratory results.

Confirm responsibility

Who is accepting the next assessment?

Separate processes

This must not be left as a routine administrative message.

Ana / in the conversation

I am calling about Lee, who reports fever and shaking after recent systemic treatment.

Dr Ford / in the conversation

Tell me the verified details and what remains unknown.

Dr Ford / in the conversation

Please preserve the symptom onset time as reported, including any uncertainty in the account.

Dr Ford / in the conversation

The last treatment date matters, but do not delay the active response while completing every detail.

Dr Ford / in the conversation

Keep this with urgent assessment, not a routine message.

Ana / in the conversation

I am confirming their timing, and we do not yet have laboratory results.

Ana / in the conversation

I will also verify the most recent treatment information from the actual record rather than rely on memory.

Ana / in the conversation

Shall I describe this as confirmed treatment toxicity in the handoff heading, or keep the cause open?

Precision in real exchanges

Purpose: Preserve an unknown
Useful wording: We do not yet have the laboratory results.

Purpose: Avoid premature diagnosis
Useful wording: I am reporting the concern, not a confirmed cause.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which opening best supports the handoff?

A. There is a small administrative question for next week.

B. All causes are already established.

C. Lee must be fine because the call is clear.

D. Urgent symptom report after recent treatment; the pathway is activated.

2. Which diagnosis cannot be asserted from this brief?

A. Confirmed neutropenia

B. Reported fever

C. Reported shaking

D. Recent treatment

3. A colleague needs the term for this meaning: Measurement of the number of neutrophils in blood. Which term fits?

A. patient preference

B. urgent assessment

C. neutrophil count

D. mucositis

D Choose the response that fits / continued

4. Rebuild the exchange from "The call cannot become a routine message". Ana: "I will also verify the most recent treatment information from the actual record rather than rely on memory." Dr Ford: [missing response] Ana: "Shall I describe this as confirmed treatment toxicity in the handoff heading, or keep the cause open?" Which response belongs in the gap?

A. Keep the causality assessment open. The symptoms followed treatment, but we have not established their cause.

B. Yes. The last treatment date matters, but do not delay the active response while completing every detail.

C. The receiving clinician must be identified through our pathway. I am accepting this clinical discussion now.

D. Correct. A neutrophil count is not available; do not fill that gap with an assumed result.

Answers and explanations, page 126

> **One useful distinction**
>
> Fever after treatment raises a concern requiring the actual clinical pathway; it does not establish neutropenia, infection, or treatment causality by itself.

E Conversation

The call cannot become a routine message

The call cannot become a routine message

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

urgent assessment / neutrophil count / escalation acknowledgment / last treatment date / causality assessment / symptom onset time / infection concern / receiving clinician / adverse event / read-back

01 **Ana:** I am calling about Lee, who reports fever and shaking after recent systemic treatment. The urgent pathway is activated.

02 **Dr Ford:** Thank you. Keep this with [1] _____, not a routine message. Tell me the verified details and what remains unknown.

03 **Ana:** The symptoms are patient-reported. I am confirming their timing, and we do not yet have laboratory results.

04 **Dr Ford:** Please preserve the [2] _____ as reported, including any uncertainty in the account.

05 **Ana:** I will also verify the most recent treatment information from the actual record rather than rely on memory.

06 **Dr Ford:** Yes. The [3] _____ matters, but do not delay the active response while completing every detail.

07 **Ana:** Shall I describe this as confirmed treatment toxicity in the handoff heading, or keep the cause open?

08 **Dr Ford:** Keep the [4] _____ open. The symptoms followed treatment, but we have not established their cause.

09 **Ana:** I have not labeled Lee neutropenic. There is no count available to support that description yet.

10 **Dr Ford:** Correct. A [5] _____ is not available; do not fill that gap with an assumed result.

Conversation continues on the next page.

The call cannot become a routine message

CONVERSATION / CONTINUED

11 **Ana:** The clinical concern remains urgent even while we avoid assigning a diagnosis that has not been established.

12 **Dr Ford:** Exactly. We can escalate an [6] _____ without declaring that infection is confirmed.

13 **Ana:** I need a clear receiving contact so that the report does not move between teams without an owner.

14 **Dr Ford:** The [7] _____ must be identified through our pathway. I am accepting this clinical discussion now.

15 **Ana:** I will repeat the patient's reported symptoms and the actions already taken, then confirm the next communication.

16 **Dr Ford:** Use [8] _____ for the important details and correct anything that differs from the record.

17 **Ana:** I will document that the concern was received and who accepted it, not simply that a call was attempted.

18 **Dr Ford:** That [9] _____ makes the handoff clear. Keep any new information with the active clinical team.

19 **Ana:** Understood. The urgent pathway remains active, the cause is unresolved, and we are not inventing missing results.

20 **Dr Ford:** Correct. Record the [10] _____ factually and follow the actual clinical and reporting procedures.

CHECK THE GAPS

1. urgent assessment / **2.** symptom onset time / **3.** last treatment date / **4.** causality assessment / **5.** neutrophil count / **6.** infection concern / **7.** receiving clinician / **8.** read-back / **9.** escalation acknowledgment / **10.** adverse event

Why these answers fit, page 127

F Check accuracy and scope

The call cannot become a routine message

Return to the case

Oncology nurse Ana calls Dr Ford about Lee, who reports fever and shaking after recent systemic treatment. The urgent oncology assessment pathway has already been activated according to the service protocol. The cause and laboratory results are unknown. The clinicians must exchange the relevant history and confirm responsibility; this case provides no triage threshold, diagnosis, or dosing instruction.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. What must the handoff confirm?

A. Only that an email was sent
B. An invented medicine dose
C. A guaranteed cause
D. The receiving clinician and responsibility

2. Which statement preserves causality correctly?

A. Treatment definitely caused every symptom.
B. Timing proves infection.
C. No assessment is needed until causality is certain.
D. Symptoms occurred after treatment; the cause is not established.

Answers and explanations, page 129

Listen for the distinction

Swap roles. Repeat the unknown-results statement and the acknowledgment without inventing a diagnosis or a dose.

Urgency and uncertainty can coexist

A clinician can request urgent assessment without claiming a confirmed cause. Lead with the concern and action already taken, then identify the information still needed.

F Say it / transfer to a new case

Confirm a received escalation

The urgent pathway is active for a new symptom report. Dr Lane accepts the handoff. Laboratory results remain pending; the cause is not established.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

active / pending / accept / established

01 **Nurse:** The urgent pathway is [1] _____.

02 **Dr Lane:** I [2] _____ the handoff.

03 **Nurse:** The laboratory results remain [3] _____.

04 **Dr Lane:** The cause is not [4] _____.

CHECK THE GAPS

1. active / **2.** accept / **3.** pending / **4.** established

Why these answers fit, page 129

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "The call cannot become a routine message". Ana: "I need a clear receiving contact so that the report does not move between teams without an owner." Dr Ford: [missing response] Ana: "I will repeat the patient's reported symptoms and the actions already taken, then confirm the next communication." Which response belongs in the gap?

A. Correct. A neutrophil count is not available; do not fill that gap with an assumed result.

B. Keep the causality assessment open. The symptoms followed treatment, but we have not established their cause.

C. The receiving clinician must be identified through our pathway. I am accepting this clinical discussion now.

D. That escalation acknowledgment makes the handoff clear. Keep any new information with the active clinical team.

Answers and explanations, page 130

A Read the situation

LESSON 06 / DISCUSSING A RESPONSE-ASSESSMENT SCAN

Explain a scan comparison while distinguishing measurement, interpretation, and an agreed treatment decision.

> **Smaller is useful, but not the whole conclusion**
>
> Pat's report describes a measured lesion as smaller than on the comparison scan. The complete response assessment is still under oncology review, and no formal response category or treatment change has been agreed. Dr Ng must acknowledge the encouraging observation without declaring cure, complete response, or a settled next regimen.

People: Pat, Patient; Dr Ng, Oncologist.

Read the record

Choose one answer per question using the facts above.

1. What does the report describe?

A. A confirmed cure
B. A new regimen already started
C. A measured lesion is smaller
D. Every lesion has disappeared

2. What is still under review?

A. Whether Pat has a report
B. Whether a comparison scan exists
C. Whether a clinician is involved
D. The complete response assessment

3. What has not been agreed?

A. That the lesion was measured
B. That Pat wants an explanation
C. A treatment change
D. That the report is available

Answers and explanations, page 131

B Vocabulary in use

CORE TERMS

response assessment
Evaluation of how disease has changed during or after treatment.

complete a response assessment

measurable lesion
Abnormality that can be measured under the relevant assessment criteria.

identify a measurable lesion

non-target lesion
Disease finding assessed outside the selected target-lesion measurements.

assess non-target lesions

complete response
Disappearance of detectable disease findings under the applicable response criteria.

qualify a complete response

progressive disease
Worsening that meets applicable progression criteria.

evaluate progressive disease

radiological response
Change in disease appearance assessed using imaging.

describe radiological response

comparison scan
Earlier imaging study used as a reference.

review the comparison scan

target lesion
Selected lesion followed for measurement under a response-assessment system.

measure a target lesion

partial response
Reduction meeting the relevant criteria without a complete response.

explain a partial response

stable disease
Disease not meeting the relevant response or progression thresholds.

discuss stable disease

new lesion
Newly identified abnormality requiring interpretation in context.

investigate a new lesion

clinical response
Change assessed through clinical symptoms, signs, or functioning.

compare clinical response

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

response assessment RESPONSE ASSESSMENT

complete a response assessment

Keep the meaning precise: Evaluation of how disease has changed during or after treatment.

partial response PARTIAL RESPONSE

explain a partial response

Keep the meaning precise: Reduction meeting the relevant criteria without a complete response.

B Precision and professional shorthand

EXTEND YOUR RANGE

remission

Reduction or disappearance of signs of disease, described as partial or complete when appropriate.

explain remission

recurrence

Return of cancer after a period of improvement or remission.

discuss recurrence

measurement variability

Differences arising from how measurements are obtained or interpreted.

account for measurement variability

scan interval

Time between imaging examinations.

confirm the scan interval

restaging

Reassessment of disease extent after treatment or a change.

explain restaging

assessment criteria

Defined rules used to classify findings or changes.

apply the relevant assessment criteria

qualified interpreter

Trained language professional supporting accurate communication between speakers.

request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

use teach-back without blame

patient preference

A person's stated wishes about care or communication.

elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

confirm the clinical handoff

B Precision and professional shorthand / continued

documented plan

Agreed next steps recorded for the people involved.

review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

preserve source attribution

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

clinical response CLINICAL RESPONSE

compare clinical response

Keep the meaning precise: Change assessed through clinical symptoms, signs, or functioning.

restaging RESTAGING

explain restaging

Keep the meaning precise: Reassessment of disease extent after treatment or a change.

C Phrases that move the work forward

State the observation

This measured lesion is smaller than on the comparison scan.

Recognize its value

That is a useful finding to discuss.

Separate the review

The complete response assessment is still under review.

Avoid overstatement

Smaller does not by itself mean cured.

Include other evidence

The team will interpret the scan with the other relevant findings.

Keep status accurate

No treatment change has been agreed yet.

Explain a category

A response category uses defined criteria, not one adjective.

Check the summary

What would you tell someone about what is known and what is still pending?

Pat / in the conversation

The report says the spot is smaller.

Pat / in the conversation

I have been waiting to hear whether I can call that good news.

Dr Ng / in the conversation

The comparison scan shows the reference point.

Dr Ng / in the conversation

This measured lesion is smaller, and that is useful information.

Pat / in the conversation

I would like to understand the finding without saying more than the report can actually support.

Dr Ng / in the conversation

The full response assessment is still under review; one measurement is not the entire conclusion.

Pat / in the conversation

Does smaller automatically mean the treatment has produced what the team calls a partial response?

Dr Ng / in the conversation

I need the full assessment before using that label for your results.

Precision in real exchanges

Purpose: Avoid overstatement
Useful wording: Smaller does not by itself mean cured.

Purpose: Include other evidence
Useful wording: The team will interpret the scan with the other relevant findings.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which update is supported?

A. One measured lesion is smaller; the full assessment remains under review.

B. Pat is cured.

C. Complete response is confirmed.

D. Every finding has resolved.

2. Which claim confuses observation and classification?

A. A comparison scan is used.

B. A lesion was measured.

C. The review is incomplete.

D. Smaller automatically means formal partial response.

3. A colleague needs the term for this meaning: Selected lesion followed for measurement under a response-assessment system. Which term fits?

A. target lesion

B. progressive disease

C. measurement variability

D. patient preference

D Choose the response that fits / continued

4. Rebuild the exchange from "Smaller is useful, but not the whole conclusion". Pat: "Does smaller automatically mean the treatment has produced what the team calls a partial response?" Dr Ng: [missing response] Pat: "I noticed the report described other findings as well. Are they part of the same review?" Which response belongs in the gap?

A. No. We must finish the review using the relevant assessment criteria and clinical context before discussing any recommendation.

B. The comparison scan shows the reference point. This measured lesion is smaller, and that is useful information.

C. That is sensible. The full response assessment is still under review; one measurement is not the entire conclusion.

D. A partial response has defined criteria. I need the full assessment before using that label for your results.

Answers and explanations, page 131

> **One useful distinction**
>
> A smaller lesion is an observation, not automatically a formal partial response. Complete response and cure are not interchangeable claims.

E Conversation

Smaller is useful, but not the whole conclusion

Smaller is useful, but not the whole conclusion

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

partial response / radiological response / target lesion / complete response / assessment criteria / Remission / Non-target lesions / response assessment / Clinical response / comparison scan

01 **Pat:** The report says the spot is smaller. I have been waiting to hear whether I can call that good news.

02 **Dr Ng:** The [1] _____ shows the reference point. This measured lesion is smaller, and that is useful information.

03 **Pat:** I would like to understand the finding without saying more than the report can actually support.

04 **Dr Ng:** That is sensible. The full [2] _____ is still under review; one measurement is not the entire conclusion.

05 **Pat:** Does smaller automatically mean the treatment has produced what the team calls a partial response?

06 **Dr Ng:** A [3] _____ has defined criteria. I need the full assessment before using that label for your results.

07 **Pat:** I noticed the report described other findings as well. Are they part of the same review?

08 **Dr Ng:** Yes. A [4] _____ is selected for measurement, but other findings can matter to the overall interpretation.

09 **Pat:** Then I should not assume that measuring one selected area means every other area has been ignored.

10 **Dr Ng:** Correct. [5] _____ and any new findings may need assessment under the appropriate system.

Conversation continues on the next page.

Smaller is useful, but not the whole conclusion

CONVERSATION / CONTINUED

11 **Pat:** My symptoms have also changed. Should I describe those even if the scan appears encouraging?

12 **Dr Ng:** Absolutely. [6] _____ and imaging findings offer different information; we need the clinical context too.

13 **Pat:** I want to tell my family there is an encouraging change, but not that everything has disappeared.

14 **Dr Ng:** That is accurate. A [7] _____ has not been established by the facts we are discussing.

15 **Pat:** And even that term would need explaining rather than being used as a simple synonym for cure?

16 **Dr Ng:** Yes. [8] _____ and cure are not interchangeable promises about an individual's future.

17 **Pat:** Does the smaller measurement mean you have already decided to change the treatment schedule?

18 **Dr Ng:** No. We must finish the review using the relevant [9] _____ and clinical context before discussing any recommendation.

19 **Pat:** Then my summary is that one lesion is smaller, the full review is pending, and the plan has not changed yet.

20 **Dr Ng:** Exactly. We will explain the [10] _____ and its implications when the complete assessment is available.

CHECK THE GAPS

1. comparison scan / **2.** response assessment / **3.** partial response / **4.** target lesion / **5.** Non-target lesions / **6.** Clinical response / **7.** complete response / **8.** Remission / **9.** assessment criteria / **10.** radiological response

Why these answers fit, page 132

F Check accuracy and scope

Smaller is useful, but not the whole conclusion

Return to the case

Pat's report describes a measured lesion as smaller than on the comparison scan. The complete response assessment is still under oncology review, and no formal response category or treatment change has been agreed. Dr Ng must acknowledge the encouraging observation without declaring cure, complete response, or a settled next regimen.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. Which treatment statement is accurate?

A. No change has been agreed.
B. A new regimen is already authorized.
C. Treatment has ended permanently.
D. The next dose is supplied here.

2. Why avoid equating complete response with cure?

A. Complete response means no review is needed.
B. They describe different claims about disease and future outcome.
C. The words have identical meanings.
D. Cure is merely a measurement unit.

Answers and explanations, page 134

Listen for the distinction

Swap roles. Repeat Pat's final summary without declaring cure or changing treatment.

Share the useful finding without expanding its meaning

Patients deserve to hear favorable observations clearly. Precision does not require withholding them; it requires separating the observation from conclusions still under review.

F Say it / transfer to a new case

A new finding awaiting interpretation

A scan mentions a new finding. The oncology review is incomplete. No progression category or treatment change has been agreed.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

new / classified / agreed / incomplete

01 **Patient:** The report describes a [1] _____ finding.

02 **Clinician:** The review remains [2] _____.

03 **Patient:** Progression is not yet [3] _____.

04 **Clinician:** No treatment change is [4] _____.

CHECK THE GAPS

1. new / **2.** incomplete / **3.** classified / **4.** agreed

Why these answers fit, page 134

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "Smaller is useful, but not the whole conclusion". Pat: "I want to tell my family there is an encouraging change, but not that everything has disappeared." Dr Ng: [missing response] Pat: "And even that term would need explaining rather than being used as a simple synonym for cure?" Which response belongs in the gap?

A. The comparison scan shows the reference point. This measured lesion is smaller, and that is useful information.

B. That is accurate. A complete response has not been established by the facts we are discussing.

C. Yes. A target lesion is selected for measurement, but other findings can matter to the overall interpretation.

D. Exactly. We will explain the radiological response and its implications when the complete assessment is available.

Answers and explanations, page 135

Review 3 / Meaning and context

Use language from the lessons you have completed. Choose one answer per question.

> **Case file**
>
> Revisit "Escalating a treatment-related symptom report" and "Discussing a response-assessment scan". Choose precise terminology, then reconstruct the missing responses using both neighboring turns. The final two questions also revisit earlier conversations.

1. A colleague needs the term for this meaning: Abnormally low neutrophil count. Which term fits?

A. extravasation
B. last treatment date
C. neutropenia
D. teach-back

2. A colleague needs the term for this meaning: Disease not meeting the relevant response or progression thresholds. Which term fits?

A. qualified interpreter
B. remission
C. stable disease
D. response assessment

3. Rebuild the exchange from "What is this option trying to achieve?". Jordan: "Could you explain the proposed schedule, then distinguish what is definite from what may change?" Dr Bell: [missing response] Jordan: "I have heard cycle used as though everyone knows what it means. Please explain that too." Which response belongs in the gap?

A. We would review the treatment regimen and the circumstances in which it might need review.
B. Your caregiving responsibility belongs in this conversation. Let us examine those demands alongside the clinical aims.
C. The treatment intent here is disease control. I am not describing it as a guaranteed cure.
D. They do not. Treatment burden includes visits and disruption as well as physical effects.

Review 3 / Meaning and context / continued

4. Rebuild the exchange from "Smaller is useful, but not the whole conclusion". Pat: "Does the smaller measurement mean you have already decided to change the treatment schedule?" Dr Ng: [missing response] Pat: "Then my summary is that one lesion is smaller, the full review is pending, and the plan has not changed yet." Which response belongs in the gap?

A. A partial response has defined criteria. I need the full assessment before using that label for your results.

B. That is sensible. The full response assessment is still under review; one measurement is not the entire conclusion.

C. The comparison scan shows the reference point. This measured lesion is smaller, and that is useful information.

D. No. We must finish the review using the relevant assessment criteria and clinical context before discussing any recommendation.

Answers and explanations, page 150

A Read the situation

LESSON 07 / EXPLAINING A CLINICAL TRIAL INVITATION

Explain research participation while separating eligibility, consent, and proven benefit.

> **An option to discuss, not an obligation**
>
> Dr Ortiz discusses a possible treatment trial with Val. Eligibility screening is not complete, and Val has not consented or enrolled. The trial compares approaches using random assignment. No investigational product, benefit probability, or payment terms are supplied. Val worries that declining would harm the relationship with the care team.

People: Val, Patient; Dr Ortiz, Oncologist.

Read the record

Choose one answer per question using the facts above.

1. What is the participation status?

A. Already enrolled
B. Assigned to a study group
C. Benefit already demonstrated for Val
D. Eligibility review incomplete; no consent or enrollment

2. How does the trial assign approaches?

A. A payment determining the group
B. Random assignment
C. Val choosing any group after enrollment
D. A guaranteed best-treatment allocation

3. What worries Val?

A. A completed treatment switch
B. Declining might harm the care relationship
C. A dose already selected here
D. A result already received

Answers and explanations, page 136

B Vocabulary in use

CORE TERMS

clinical trial
Research study evaluating a health intervention in people.

discuss a clinical trial

eligibility criteria
Requirements determining whether a person may enter a study.

review eligibility criteria

screening visit
Assessment checking whether trial-entry requirements are met.

arrange a screening visit

enrollment
Formal entry into a study after required processes.

distinguish enrollment from screening

randomization
Assignment to study groups through a chance-based process.

explain randomization

study arm
Group receiving a specified intervention or approach in a trial.

describe a study arm

control group
Comparison group used to evaluate an intervention.

explain the control group

investigational treatment
Treatment being studied for the relevant use.

discuss an investigational treatment

standard of care
Accepted clinical approach for the relevant setting and circumstances.

compare with standard of care

research protocol
Approved plan specifying how a study is conducted.

explain the research protocol

primary endpoint
Main outcome used to evaluate the study question.

identify the primary endpoint

overall survival
Time from a defined starting point until death from any cause.

interpret overall survival

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

clinical trial CLINICAL TRIAL

discuss a clinical trial

Keep the meaning precise: Research study evaluating a health intervention in people.

study arm STUDY ARM

describe a study arm

Keep the meaning precise: Group receiving a specified intervention or approach in a trial.

B Precision and professional shorthand

EXTEND YOUR RANGE

progression-free survival

Time from a defined point without disease progression or death under study definitions.

explain progression-free survival

withdrawal

Ending participation through the applicable study process.

explain withdrawal options

research coordinator

Professional organizing study visits and participation processes.

contact the research coordinator

qualified interpreter

Trained language professional supporting accurate communication between speakers.

request a qualified interpreter

patient preference

A person's stated wishes about care or communication.

elicit the patient preference

documented plan

Agreed next steps recorded for the people involved.

review the documented plan

voluntary participation

Taking part by choice without improper pressure.

protect voluntary participation

consent discussion

Conversation supporting an informed decision about participation.

allow time for the consent discussion

therapeutic misconception

Mistaken belief that research is designed solely to provide individualized treatment benefit.

address therapeutic misconception

teach-back

Checking an explanation by inviting the listener to restate its meaning.

use teach-back without blame

clinical handoff

Transfer of relevant care information and responsibility between professionals.

confirm the clinical handoff

source attribution

Identification of who reported, observed, or established a fact.

preserve source attribution

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

overall survival OVERALL SURVIVAL

interpret overall survival

Keep the meaning precise: Time from a defined starting point until death from any cause.

research coordinator RESEARCH COORDINATOR

contact the research coordinator

Keep the meaning precise: Professional organizing study visits and participation processes.

C Phrases that move the work forward

Introduce research

This is an option to discuss, not an obligation.

Explain assignment

Random assignment determines the study group.

Protect the relationship

Declining research does not remove your right to appropriate care.

Avoid a guarantee

An investigational treatment is not proven better simply because it is new.

Val / in the conversation

I worry that saying no would make you think I am not cooperating with care.

Val / in the conversation

Am I already accepted because you brought it up, or is there another process before that?

Dr Ortiz / in the conversation

Discussing the study does not establish that you can enter it.

Dr Ortiz / in the conversation

The research protocol describes its question, procedures, and participant requirements.

State uncertainty

The study is asking a question that is not already settled.

Separate stages

Eligibility review is not the same as enrollment.

Invite review

Take the opportunity to ask about visits, risks, and alternatives.

Clarify withdrawal

We will explain the actual withdrawal process and remaining care arrangements.

Dr Ortiz / in the conversation

This clinical trial is an option to discuss.

Dr Ortiz / in the conversation

We still need to review the eligibility criteria.

Val / in the conversation

I would like to understand what the study is trying to find out before considering any forms.

Dr Ortiz / in the conversation

We will review the relevant details with you.

Precision in real exchanges

Purpose: Separate stages
Useful wording: Eligibility review is not the same as enrollment.

Purpose: Protect the relationship
Useful wording: Declining research does not remove your right to appropriate care.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which statement separates the stages correctly?

A. Discussing a trial automatically enrolls Val.

B. Screening proves benefit.

C. A consent form guarantees entry.

D. Eligibility is under review; Val is not enrolled.

2. Which claim creates therapeutic misconception?

A. Participation is voluntary.

B. Random assignment determines a group.

C. The research exists solely to give Val the best individual treatment.

D. The study has a research question.

3. A colleague needs the term for this meaning: Formal entry into a study after required processes. Which term fits?

A. standard of care

B. enrollment

C. patient preference

D. withdrawal

D Choose the response that fits / continued

4. Rebuild the exchange from "An option to discuss, not an obligation". Val: "I would like to understand what the study is trying to find out before considering any forms." Dr Ortiz: [missing response] Val: "Would you choose whichever treatment group you think is best for me after I join?" Which response belongs in the gap?

A. An investigational treatment is being studied; its possible advantages and disadvantages are not a promised result for you.

B. This clinical trial is an option to discuss. Declining research does not remove your right to appropriate care.

C. The research protocol describes its question, procedures, and participant requirements. We will review the relevant details with you.

D. We still need to review the eligibility criteria. Discussing the study does not establish that you can enter it.

Answers and explanations, page 136

> **One useful distinction**
>
> Trial eligibility is not enrollment, and consent does not guarantee benefit. Randomization is not a clinician choosing the group believed best for one participant.

E Conversation

An option to discuss, not an obligation

An option to discuss, not an obligation

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

standard of care / research coordinator / randomization / investigational treatment / Enrollment / research protocol / consent discussion / withdrawal / clinical trial / eligibility criteria

01 **Val:** You mentioned a trial. I worry that saying no would make you think I am not cooperating with care.

02 **Dr Ortiz:** This [1] _____ is an option to discuss. Declining research does not remove your right to appropriate care.

03 **Val:** Am I already accepted because you brought it up, or is there another process before that?

04 **Dr Ortiz:** We still need to review the [2] _____. Discussing the study does not establish that you can enter it.

05 **Val:** I would like to understand what the study is trying to find out before considering any forms.

06 **Dr Ortiz:** The [3] _____ describes its question, procedures, and participant requirements. We will review the relevant details with you.

07 **Val:** Would you choose whichever treatment group you think is best for me after I join?

08 **Dr Ortiz:** This study uses [4] _____. Assignment follows a chance-based process rather than my choosing an arm for you.

09 **Val:** That matters to my decision. I do not want to assume that the new option is already known to be better.

10 **Dr Ortiz:** An [5] _____ is being studied; its possible advantages and disadvantages are not a promised result for you.

Conversation continues on the next page.

An option to discuss, not an obligation

CONVERSATION / CONTINUED

11 **Val:** Please compare participation with the ordinary care options I could discuss outside the study.

12 **Dr Ortiz:** We should explain the relevant [6] _____ and other reasonable alternatives, not present research as the only possible conversation.

13 **Val:** The visits and tests may affect my work. Can I ask about those before deciding whether to proceed?

14 **Dr Ortiz:** Certainly. The [7] _____ can help explain practical arrangements, alongside our discussion of the clinical questions.

15 **Val:** If I first agree and later want to stop, I need to understand what happens to my care.

16 **Dr Ortiz:** We will explain [8] _____ using the actual study process and clarify the remaining care arrangements.

17 **Val:** I would like time to read the information and return with questions rather than sign because I feel rushed.

18 **Dr Ortiz:** That is part of a proper [9] _____. Questions and time to understand are important to a voluntary decision.

19 **Val:** Then eligibility is still being checked, and I have not consented or joined by having this conversation.

20 **Dr Ortiz:** Correct. [10] _____ has not happened. We can continue discussing the option without treating today as a commitment.

CHECK THE GAPS

1. clinical trial / **2.** eligibility criteria / **3.** research protocol / **4.** randomization / **5.** investigational treatment / **6.** standard of care / **7.** research coordinator / **8.** withdrawal / **9.** consent discussion / **10.** Enrollment

Why these answers fit, page 137

F Check accuracy and scope

An option to discuss, not an obligation

> **Return to the case**
>
> Dr Ortiz discusses a possible treatment trial with Val. Eligibility screening is not complete, and Val has not consented or enrolled. The trial compares approaches using random assignment. No investigational product, benefit probability, or payment terms are supplied. Val worries that declining would harm the relationship with the care team.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. What does randomization mean?

A. A completed response assessment
B. Chance-based assignment under the study process
C. The doctor selecting a guaranteed best arm
D. The patient purchasing a preferred arm

2. Which response to Val's concern is appropriate?

A. Signing now is the only way to ask questions.
B. Declining research does not remove the right to appropriate care.
C. The team will stop listening if you decline.
D. You owe the study your participation.

Answers and explanations, page 139

> **Listen for the distinction**
>
> Swap roles. Repeat the randomization explanation and the response to perceived pressure.

> **Keep research and care roles clear**
>
> An invitation can feel like a recommendation the patient must obey. Explain the research purpose, uncertainty, and voluntary decision without presenting an experimental option as proven superior.

F Say it / transfer to a new case

A screening visit is not enrollment

Jo has agreed to a trial-screening visit. Eligibility remains unknown. Jo has not enrolled and wants the withdrawal information explained before making a participation decision.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

withdrawal / enrolled / screening / unknown

01 **Jo:** I agreed to a [1] _____ visit.

02 **Clinician:** Your eligibility remains [2] _____.

03 **Jo:** I have not [3] _____ in the study.

04 **Clinician:** We will explain the [4] _____ process.

CHECK THE GAPS

1. screening / **2.** unknown / **3.** enrolled / **4.** withdrawal

Why these answers fit, page 139

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "An option to discuss, not an obligation". Val: "The visits and tests may affect my work. Can I ask about those before deciding whether to proceed?" Dr Ortiz: [missing response] Val: "If I first agree and later want to stop, I need to understand what happens to my care." Which response belongs in the gap?

A. This study uses randomization. Assignment follows a chance-based process rather than my choosing an arm for you.

B. Correct. Enrollment has not happened. We can continue discussing the option without treating today as a commitment.

C. We will explain withdrawal using the actual study process and clarify the remaining care arrangements.

D. Certainly. The research coordinator can help explain practical arrangements, alongside our discussion of the clinical questions.

Answers and explanations, page 140

A Read the situation

LESSON 08 / DISCUSSING PALLIATIVE CARE AND PRIORITIES

Explain a supportive-care referral and elicit goals without equating it with abandonment or an automatic change in treatment.

> **Support alongside treatment**
>
> Dr Ahmed offers Rowan a palliative-care consultation for symptoms and distress while an anticancer-treatment discussion continues. Rowan hears the referral as a decision to stop treatment. No decision to stop treatment, enter hospice, or set a resuscitation status is supplied. Rowan wants more comfortable time with family and clearer help with symptoms.

People: Rowan, Patient; Dr Ahmed, Oncologist.

Read the record

Choose one answer per question using the facts above.

1. Why is the consultation offered?

A. A completed resuscitation decision
B. Symptoms and distress
C. A supplied decision to stop all treatment
D. An automatic hospice admission

2. What does Rowan fear?

A. That a trial has enrolled them
B. That treatment is being stopped
C. That the report contains a new grade
D. That a scan is already normal

3. Which preference is stated?

A. More comfortable time with family
B. A particular resuscitation status
C. A guaranteed survival period
D. An instruction to refuse all services

Answers and explanations, page 141

B Vocabulary in use

CORE TERMS

palliative care
Care addressing symptoms, distress, and quality of life in serious illness.

introduce palliative care

hospice care
Care focused on comfort near the end of life under applicable service criteria.

distinguish hospice care

symptom management
Assessment and treatment of distressing symptoms.

coordinate symptom management

goals-of-care discussion
Conversation about priorities and how care should respond to them.

open a goals-of-care discussion

serious-illness conversation
Discussion of understanding, concerns, values, and care in serious illness.

structure a serious-illness conversation

advance care planning
Process of discussing and recording preferences for future care.

offer advance care planning

advance directive
Document expressing future-care wishes or decision arrangements under applicable law.

review an advance directive

surrogate decision-maker
Person authorized to decide when the patient cannot under applicable rules.

clarify the surrogate decision-maker

resuscitation status
Documented decision or order about resuscitation in the relevant setting.

clarify resuscitation status

comfort-focused care
Care organized primarily around comfort and relief of distress.

explain comfort-focused care

concurrent care
Care approaches provided alongside one another.

discuss concurrent care

care preference
Person's expressed wishes about aspects of care.

elicit a care preference

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

palliative care PALLIATIVE CARE

introduce palliative care

Keep the meaning precise: Care addressing symptoms, distress, and quality of life in serious illness.

advance care planning ADVANCE CARE PLANNING

offer advance care planning

Keep the meaning precise: Process of discussing and recording preferences for future care.

B Precision and professional shorthand

EXTEND YOUR RANGE

family meeting

Planned discussion involving the patient and appropriate family or support people.

arrange a family meeting

spiritual concern

Concern about meaning, belief, connection, or existential distress.

invite a spiritual concern

interdisciplinary support

Help provided by professionals with complementary care roles.

coordinate interdisciplinary support

qualified interpreter

Trained language professional supporting accurate communication between speakers.

request a qualified interpreter

patient preference

A person's stated wishes about care or communication.

elicit the patient preference

documented plan

Agreed next steps recorded for the people involved.

review the documented plan

caregiver strain

Physical or emotional burden experienced by someone providing care.

acknowledge caregiver strain

symptom burden

Combined effect of symptoms on the person's life.

assess symptom burden

values clarification

Exploration of what matters to a person in a care decision.

support values clarification

teach-back

Checking an explanation by inviting the listener to restate its meaning.

use teach-back without blame

clinical handoff

Transfer of relevant care information and responsibility between professionals.

confirm the clinical handoff

source attribution

Identification of who reported, observed, or established a fact.

preserve source attribution

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

care preference CARE PREFERENCE

elicit a care preference

Keep the meaning precise: Person's expressed wishes about aspects of care.

interdisciplinary support INTERDISCIPLINARY SUPPORT

coordinate interdisciplinary support

Keep the meaning precise: Help provided by professionals with complementary care roles.

C Phrases that move the work forward

Ask what was heard

What did the phrase palliative care suggest to you?

Explain the purpose

The team can help with symptoms and distress alongside other care.

Separate decisions

Hospice and resuscitation decisions require their own discussions.

Avoid abandonment

We will continue to address your care needs.

Rowan / in the conversation

When you mentioned palliative care, I heard that you were giving up on the cancer treatment.

Dr Ahmed / in the conversation

I want to understand that concern before explaining the referral.

Dr Ahmed / in the conversation

This is an offer of concurrent care.

Rowan / in the conversation

Then please tell me what the additional team would actually help with in everyday life.

Correct the inference

This referral is not a decision to stop treatment.

Elicit a priority

What would make the coming weeks more manageable for you?

Invite support

We can discuss who you want involved in a family meeting.

Make goals concrete

More comfortable time with family is a priority we should keep visible.

Dr Ahmed / in the conversation

What did palliative care suggest to you?

Rowan / in the conversation

I thought it meant every other treatment would stop, and that I would no longer see this team.

Dr Ahmed / in the conversation

It is not a decision to stop treatment or to stop addressing your care needs.

Dr Ahmed / in the conversation

They can help with symptom management, distress, and the effects of serious illness on you and those supporting you.

Precision in real exchanges

Purpose: Elicit a priority
Useful wording: What would make the coming weeks more manageable for you?

Purpose: Separate decisions
Useful wording: Hospice and resuscitation decisions require their own discussions.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which statement explains the referral correctly?

A. Palliative care can support symptoms alongside treatment.
B. The referral automatically stops anticancer treatment.
C. The referral determines resuscitation status.
D. The referral guarantees hospice admission.

2. Which question explores the misunderstanding?

A. Why did you misunderstand an obvious term?
B. You agree it means giving up?
C. Can we avoid your concern?
D. What did the phrase suggest to you?

3. A colleague needs the term for this meaning: Conversation about priorities and how care should respond to them. Which term fits?

A. spiritual concern
B. patient preference
C. goals-of-care discussion
D. resuscitation status

D Choose the response that fits / continued

4. Rebuild the exchange from "Support alongside treatment". Rowan: "Then please tell me what the additional team would actually help with in everyday life." Dr Ahmed: [missing response] Rowan: "I want more comfortable time with my family. At present the symptoms take over the entire day." Which response belongs in the gap?

A. They can help with symptom management, distress, and the effects of serious illness on you and those supporting you.

B. What did palliative care suggest to you? I want to understand that concern before explaining the referral.

C. We can discuss a family meeting and who you want involved, keeping your priorities and permissions clear.

D. We should clarify that. A surrogate decision-maker has a specific role under the applicable arrangements, not simply because a relative attends.

Answers and explanations, page 141

> **One useful distinction**
>
> Palliative care can accompany anticancer treatment. A referral does not automatically mean hospice enrollment, treatment cessation, or a resuscitation decision.

E Conversation

Support alongside treatment

Support alongside treatment

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

goals-of-care discussion / surrogate decision-maker / Hospice care / Caregiver strain / palliative care / symptom management / concurrent care / family meeting / resuscitation status / symptom burden

01 **Rowan:** When you mentioned palliative care, I heard that you were giving up on the cancer treatment.

02 **Dr Ahmed:** What did [1] _____ suggest to you? I want to understand that concern before explaining the referral.

03 **Rowan:** I thought it meant every other treatment would stop, and that I would no longer see this team.

04 **Dr Ahmed:** This is an offer of [2] _____. It is not a decision to stop treatment or to stop addressing your care needs.

05 **Rowan:** Then please tell me what the additional team would actually help with in everyday life.

06 **Dr Ahmed:** They can help with [3] _____, distress, and the effects of serious illness on you and those supporting you.

07 **Rowan:** I want more comfortable time with my family. At present the symptoms take over the entire day.

08 **Dr Ahmed:** That [4] _____ matters. We should keep comfortable family time visible when we discuss care options.

09 **Rowan:** Does agreeing to this consultation mean I have agreed to hospice without realizing it?

10 **Dr Ahmed:** No. [5] _____ is a separate discussion with its own circumstances and service criteria.

Conversation continues on the next page.

Support alongside treatment

CONVERSATION / CONTINUED

11 **Rowan:** I also do not want one referral to become a decision about resuscitation without anyone asking me.

12 **Dr Ahmed:** It does not establish your [6] _____. Those decisions require their own appropriate discussion and documentation.

13 **Rowan:** My daughter helps a great deal, but I do not want everyone to assume she makes all decisions for me.

14 **Dr Ahmed:** We should clarify that. A [7] _____ has a specific role under the applicable arrangements, not simply because a relative attends.

15 **Rowan:** Could we have a conversation together where she can hear what I want without taking over the discussion?

16 **Dr Ahmed:** We can discuss a [8] _____ and who you want involved, keeping your priorities and permissions clear.

17 **Rowan:** I also want someone to notice how exhausted she is. She keeps saying she is fine.

18 **Dr Ahmed:** [9] _____ is worth discussing, with her involvement and the appropriate support.

19 **Rowan:** I understand better now. The referral adds symptom support; it does not silently settle every future decision.

20 **Dr Ahmed:** Exactly. A [10] _____ keeps those priorities explicit while we continue the other necessary clinical conversations.

CHECK THE GAPS

1. palliative care / **2.** concurrent care / **3.** symptom management / **4.** symptom burden / **5.** Hospice care / **6.** resuscitation status / **7.** surrogate decision-maker / **8.** family meeting / **9.** Caregiver strain / **10.** goals-of-care discussion

Why these answers fit, page 142

F Check accuracy and scope

Support alongside treatment

> **Return to the case**
>
> Dr Ahmed offers Rowan a palliative-care consultation for symptoms and distress while an anticancer-treatment discussion continues. Rowan hears the referral as a decision to stop treatment. No decision to stop treatment, enter hospice, or set a resuscitation status is supplied. Rowan wants more comfortable time with family and clearer help with symptoms.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. What priority should remain visible?

A. Comfortable time with family
B. A resuscitation choice invented here
C. A guaranteed prognosis
D. A decision attributed to an absent relative

2. Which assumption is unsafe?

A. Preferences need discussion.
B. Symptoms need attention.
C. Different care decisions should be separated.
D. Every relative present has automatic decision-making authority.

Answers and explanations, page 144

> **Listen for the distinction**
>
> Swap roles. Repeat the response to Rowan's fear without promising an outcome or assigning a resuscitation status.

> **Explore the meaning the patient heard**
>
> Terms associated with serious illness can carry meanings beyond their clinical definition. Ask what the referral sounded like before correcting the misunderstanding.

F Say it / transfer to a new case

Name a concrete care priority

Taylor wants symptom support while treatment options are reviewed. Taylor would like friend Jo at a meeting but has not named Jo as a legally authorized decision-maker.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

Jo / review / symptoms / authority

01 **Taylor:** I want help with my [1] _____.

02 **Clinician:** Treatment options remain under [2] _____.

03 **Taylor:** I would like [3] _____ at the meeting.

04 **Clinician:** Attendance does not establish decision-making [4] _____.

CHECK THE GAPS

1. symptoms / **2.** review / **3.** Jo / **4.** authority

Why these answers fit, page 144

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "Support alongside treatment". Rowan: "My daughter helps a great deal, but I do not want everyone to assume she makes all decisions for me." Dr Ahmed: [missing response] Rowan: "Could we have a conversation together where she can hear what I want without taking over the discussion?" Which response belongs in the gap?

A. What did palliative care suggest to you? I want to understand that concern before explaining the referral.

B. We should clarify that. A surrogate decision-maker has a specific role under the applicable arrangements, not simply because a relative attends.

C. They can help with symptom management, distress, and the effects of serious illness on you and those supporting you.

D. This is an offer of concurrent care. It is not a decision to stop treatment or to stop addressing your care needs.

Answers and explanations, page 145

Review 4 / Meaning and context

Use language from the lessons you have completed. Choose one answer per question.

> **Case file**
>
> Revisit "Explaining a clinical trial invitation" and "Discussing palliative care and priorities". Choose precise terminology, then reconstruct the missing responses using both neighboring turns. The final two questions also revisit earlier conversations.

1. A colleague needs the term for this meaning: Assessment checking whether trial-entry requirements are met. Which term fits?

A. teach-back
B. voluntary participation
C. screening visit
D. investigational treatment

2. A colleague needs the term for this meaning: Person authorized to decide when the patient cannot under applicable rules. Which term fits?

A. surrogate decision-maker
B. palliative care
C. family meeting
D. qualified interpreter

3. Rebuild the exchange from "Smaller is useful, but not the whole conclusion". Pat: "Then I should not assume that measuring one selected area means every other area has been ignored." Dr Ng: [missing response] Pat: "My symptoms have also changed. Should I describe those even if the scan appears encouraging?" Which response belongs in the gap?

A. Correct. Non-target lesions and any new findings may need assessment under the appropriate system.
B. Absolutely. Clinical response and imaging findings offer different information; we need the clinical context too.
C. Yes. Remission and cure are not interchangeable promises about an individual's future.
D. That is accurate. A complete response has not been established by the facts we are discussing.

Review 4 / Meaning and context / continued

4. Rebuild the exchange from "Support alongside treatment". Rowan: "I also want someone to notice how exhausted she is. She keeps saying she is fine." Dr Ahmed: [missing response] Rowan: "I understand better now. The referral adds symptom support; it does not silently settle every future decision." Which response belongs in the gap?

A. Caregiver strain is worth discussing, with her involvement and the appropriate support.

B. No. Hospice care is a separate discussion with its own circumstances and service criteria.

C. It does not establish your resuscitation status. Those decisions require their own appropriate discussion and documentation.

D. That symptom burden matters. We should keep comfortable family time visible when we discuss care options.

Answers and explanations, page 152

Language for the next case

Preparing a second-opinion record

pathology report

The pathology report contains the relevant tissue-assessment findings and is more specific than an abbreviated diagnosis in an appointment letter.

Dr Hart: Yes. The pathology report contains the relevant findings, not merely the abbreviated label in a letter.

imaging studies

Imaging studies include the acquired examinations used for review and are distinct from the written summaries describing them.

Dr Hart: The reviewing team may need the actual imaging studies as well as the reports describing them.

treatment summary

A treatment summary records relevant prior therapy and must be checked across sites rather than assumed complete from one institution's list.

Dr Hart: We should verify the treatment summary across the relevant records, including what remains unconfirmed.

record-transfer authorization

Record-transfer authorization defines the appropriate permission for sending specified information to the receiving service through the approved process.

Dr Hart: The record-transfer authorization should identify the relevant information and recipient through the approved process.

referral packet

A referral packet is the collection of relevant documents and materials required by the receiving service and may need more than one uploaded file.

Dr Hart: That is useful. A referral packet can be incomplete even when some documents have arrived.

Appointment confirmation

Appointment confirmation establishes an actual scheduled review and is separate from a request being sent or records being received.

Dr Hart: Exactly. Appointment confirmation and the eventual clinical recommendation remain separate steps.

Read the phrases aloud. Then complete the conversation on the following pages using the word bank.

Conversation

Preparing a second-opinion record

Patient Mira asks Dr Hart for a second opinion. The receiving service requests the pathology report, relevant imaging, and treatment summary, with authorized transfer arrangements. The clinician supports the request. No new treatment recommendation or review appointment is confirmed.

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

pathology report / record-transfer authorization / treatment summary / referral packet / Appointment confirmation / imaging studies

01 **Mira:** I would like a second opinion, but I worry that asking will sound as though I do not trust you.

02 **Dr Hart:** You can ask for another review. Let us discuss the information the receiving service needs and the transfer process.

03 **Mira:** They requested a pathology report. Is that different from the short diagnosis line in my appointment letter?

04 **Dr Hart:** Yes. The [1] _____ contains the relevant findings, not merely the abbreviated label in a letter.

05 **Mira:** They also asked for the images, not only the written scan report. Why would both be requested?

06 **Dr Hart:** The reviewing team may need the actual [2] _____ as well as the reports describing them.

07 **Mira:** I have had treatment at two sites. I do not want the second team to assume the list from one site is complete.

08 **Dr Hart:** We should verify the [3] _____ across the relevant records, including what remains unconfirmed.

09 **Mira:** Does sending all the records mean the second team will definitely recommend something different?

10 **Dr Hart:** No. A second review may agree, differ, or need further information; we cannot promise its conclusion.

Conversation continues on the next page.

Preparing a second-opinion record

CONVERSATION / CONTINUED

11 **Mira:** I would like to know what I am authorizing when the records are transferred.

12 **Dr Hart:** The [4] _____ should identify the relevant information and recipient through the approved process.

13 **Mira:** If a specimen is requested for review, is that automatically included with an electronic report?

14 **Dr Hart:** Not necessarily. Any specimen or slide request has its own actual handling and transfer arrangements.

15 **Mira:** I will ask the receiving service what is still missing instead of assuming that one uploaded file completes everything.

16 **Dr Hart:** That is useful. A [5] _____ can be incomplete even when some documents have arrived.

17 **Mira:** Who will tell me whether the review appointment is actually confirmed?

18 **Dr Hart:** We should identify the receiving contact and distinguish receipt of records from an accepted appointment.

19 **Mira:** Then I can ask for a review without treating the transfer itself as a new treatment plan.

20 **Dr Hart:** Exactly. [6] _____ and the eventual clinical recommendation remain separate steps.

CHECK THE GAPS

1. pathology report / **2.** imaging studies / **3.** treatment summary / **4.** record-transfer authorization / **5.** referral packet / **6.** Appointment confirmation

Why these answers fit, page 154

The images have not arrived

The receiving team has the pathology report and treatment summary but not the imaging studies. No appointment is confirmed.

Complete the exchange using each phrase once. Read both roles, then switch roles and repeat with the bank covered.

WORD AND PHRASE BANK

missing / arrived / complete / confirmed

01 **Patient:** The imaging studies are still [1] _____.

02 **Clinician:** The report and summary have [2] _____.

03 **Patient:** The referral packet is not [3] _____.

04 **Clinician:** The appointment is not yet [4] _____.

CHECK THE GAPS

1. missing / **2.** arrived / **3.** complete / **4.** confirmed

Why these answers fit, page 154

Keep the distinction audible

Support a second opinion and distinguish a complete referral packet from a guaranteed recommendation.

Language for the next case

An oncology handoff after hospital discharge

discharge summary

The discharge summary records the hospital episode and discharge instructions but must be checked against the actual current plan and medicine information.

Dr Sen: The discharge summary is available, but I want to confirm that the current version reached your team.

medication discrepancy

A medication discrepancy is a conflict between medicine records or instructions that requires verification rather than a guess about which version applies.

Dr Ford: That creates a medication discrepancy requiring verification. The language of the handoff must not imply that both lists are simultaneously correct.

treatment hold

A treatment hold indicates an interruption under the actual plan and does not itself specify when or whether treatment should resume.

Dr Ford: A treatment hold and a restart decision are different. We need the responsible oncology assessment.

result ownership

Result ownership identifies who must review and act on a pending result so it is not lost between inpatient and outpatient services.

Dr Ford: Let us assign result ownership instead of assuming that whichever team sees it first will take responsibility.

reconciled plan

A reconciled plan aligns verified instructions and responsibilities after discrepancies are resolved rather than combining conflicting records without review.

Dr Ford: A reconciled plan needs verified instructions, not a merged list that preserves contradictions.

handoff acknowledgment

Handoff acknowledgment confirms that the receiving team obtained the information and accepted the relevant responsibilities rather than merely receiving a file.

Dr Ford: We will provide handoff acknowledgment, keeping discharge, pending results, and treatment decisions distinct.

Read the phrases aloud. Then complete the conversation on the following pages using the word bank.

Conversation

An oncology handoff after hospital discharge

Hospital clinician Dr Sen hands over patient Jules to oncologist Dr Ford after discharge. A medicine list was changed in hospital, but the outpatient record still shows an earlier list. The clinicians will verify the actual discharge instructions and assign follow-up. No medicine names, doses, or authority to restart treatment are supplied.

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

medication discrepancy / reconciled plan / result ownership / treatment hold / handoff acknowledgment / discharge summary

01 **Dr Sen:** Jules has been discharged, and the hospital medicine list differs from the earlier outpatient list. We need to reconcile them.

02 **Dr Ford:** Please identify the verified changes and their source rather than asking the patient to choose between two conflicting lists.

03 **Dr Sen:** The [1] _____ is available, but I want to confirm that the current version reached your team.

04 **Dr Ford:** We will acknowledge receipt and review the actual instructions before presenting the earlier list as current.

05 **Dr Sen:** Some changes relate to the hospital episode, and the outpatient record has not yet been updated.

06 **Dr Ford:** That creates a [2] _____ requiring verification. The language of the handoff must not imply that both lists are simultaneously correct.

Conversation / continued

07 **Dr Sen:** I will distinguish a documented instruction from a suggestion that still needs the responsible clinician's decision.

08 **Dr Ford:** Good. We also need the reason for each verified change, where the actual record provides it.

09 **Dr Sen:** A note mentions treatment being held, but the wording alone does not authorize restarting it today.

10 **Dr Ford:** A [3] _____ and a restart decision are different. We need the responsible oncology assessment.

Conversation continues on the next page.

An oncology handoff after hospital discharge

CONVERSATION / CONTINUED

11 **Dr Sen:** The patient is worried that discharge means every cancer-care question has already been settled.

12 **Dr Ford:** Discharge does not automatically resolve the outpatient plan. We should explain what is confirmed and what remains open.

13 **Dr Sen:** One result is still pending, and its reviewing clinician needs to be identified explicitly.

14 **Dr Ford:** Let us assign [4] _____ instead of assuming that whichever team sees it first will take responsibility.

15 **Dr Sen:** I will include the appropriate contact for questions about the hospital changes.

16 **Dr Ford:** And our team will confirm the oncology follow-up arrangements through the actual service process.

17 **Dr Sen:** The patient should receive one clear explanation once the conflicting information is resolved.

18 **Dr Ford:** A [5] _____ needs verified instructions, not a merged list that preserves contradictions.

19 **Dr Sen:** I will send the current summary and confirm that the outstanding tasks have named owners.

20 **Dr Ford:** We will provide [6] _____, keeping discharge, pending results, and treatment decisions distinct.

CHECK THE GAPS

1. discharge summary / **2.** medication discrepancy / **3.** treatment hold / **4.** result ownership / **5.** reconciled plan / **6.** handoff acknowledgment

Why these answers fit, page 156

A pending result needs an owner

The discharge summary lists a pending result but no reviewing clinician. The two teams clarify who will review it. No result is yet available.

Complete the exchange using each phrase once. Read both roles, then switch roles and repeat with the bank covered.

WORD AND PHRASE BANK

named / ownership / pending / result

01 **Hospital clinician:** The result is still [1] _____.

02 **Oncologist:** The reviewing clinician was not [2] _____.

03 **Hospital clinician:** We need explicit [3] _____.

04 **Oncologist:** No conclusion can be drawn from an unavailable [4] _____.

CHECK THE GAPS

1. pending / **2.** named / **3.** ownership / **4.** result

Why these answers fit, page 156

Keep the distinction audible

Reconcile inpatient changes with the oncology plan and assign unresolved questions.

Language for the next case

Discussing the financial burden of treatment

cost-related barrier

A cost-related barrier is a practical difficulty affecting access and is not equivalent to an informed refusal of treatment.

Dr Bell: A cost-related barrier is different from refusing treatment. We should document what is actually making attendance difficult.

out-of-pocket costs

Out-of-pocket costs are amounts the patient may need to pay under the actual coverage arrangements and cannot be guaranteed from an unverified estimate.

Dr Bell: The appropriate team can help clarify out-of-pocket costs under your actual coverage, without promising a figure before verification.

patient navigation

Patient navigation helps coordinate practical access and support across services without replacing the clinical plan or guaranteeing eligibility.

Dr Bell: With your permission, we can involve patient navigation or the relevant support service to explore those options.

Eligibility review

Eligibility review checks whether the program's actual criteria are met and is separate from submitting an application or receiving approval.

Dr Bell: No. Eligibility review and approval remain separate steps, and we should be clear about what is still pending.

treatment tradeoff

A treatment tradeoff concerns the benefits, harms, and practical implications of actual options and requires an appropriate clinical discussion rather than an automatic cost-based substitution.

Dr Bell: No. A treatment tradeoff requires a proper clinical discussion; no substitution is being made in this conversation.

Financial toxicity

Financial toxicity describes the harmful financial burden associated with care and deserves attention without blaming the patient or promising unverified assistance.

Dr Bell: Exactly. Financial toxicity deserves attention, with honest limits on what we can confirm and a clear next step.

Read the phrases aloud. Then complete the conversation on the following pages using the word bank.

Conversation

Discussing the financial burden of treatment

Patient Rowan tells Dr Bell that transport costs and unpaid time away from work may interfere with appointments. Dr Bell asks permission to involve the support team and discusses verifying actual coverage and assistance options. No insurance approval, eligibility decision, or treatment substitution is supplied.

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

treatment tradeoff / out-of-pocket costs / Eligibility review / cost-related barrier / patient navigation / Financial toxicity

01 **Rowan:** I want to keep my appointments, but transport and unpaid time away from work are becoming difficult to manage.

02 **Dr Bell:** Thank you for telling me. Those barriers belong in the care discussion, not in a judgment about your commitment.

03 **Rowan:** I was afraid that mentioning money would be recorded as refusing the treatment.

04 **Dr Bell:** A [1] _____ is different from refusing treatment. We should document what is actually making attendance difficult.

05 **Rowan:** The hospital estimate also uses terms I do not understand, and I do not know which amount I might owe.

06 **Dr Bell:** The appropriate team can help clarify [2] _____ under your actual coverage, without promising a figure before verification.

07 **Rowan:** Could someone help with transport or finding out whether an assistance program is relevant?

08 **Dr Bell:** With your permission, we can involve [3] _____ or the relevant support service to explore those options.

09 **Rowan:** I would like that, but I do not want my employer contacted automatically.

10 **Dr Bell:** We should agree on the information-sharing permissions and recipients rather than treat a support referral as unlimited consent.

Conversation continues on the next page.

Discussing the financial burden of treatment

CONVERSATION / CONTINUED

11 **Rowan:** If an assistance application is submitted, does that mean the cost will definitely be covered?

12 **Dr Bell:** No. [4] _____ and approval remain separate steps, and we should be clear about what is still pending.

13 **Rowan:** I can provide the documents the team actually needs, but I may need help understanding the forms.

14 **Dr Bell:** We can ask what language and administrative support is available through the real service.

15 **Rowan:** Does telling you about the cost mean you will change treatment without discussing it with me?

16 **Dr Bell:** No. A [5] _____ requires a proper clinical discussion; no substitution is being made in this conversation.

17 **Rowan:** I would feel better with one contact who can explain what has been applied for and what is confirmed.

18 **Dr Bell:** Let us identify that contact and keep applications, estimates, and confirmed arrangements clearly distinguished.

19 **Rowan:** Then the practical difficulty can be addressed without assuming that I have chosen to stop care.

20 **Dr Bell:** Exactly. [6] _____ deserves attention, with honest limits on what we can confirm and a clear next step.

CHECK THE GAPS

1. cost-related barrier / **2.** out-of-pocket costs / **3.** patient navigation / **4.** Eligibility review / **5.** treatment tradeoff / **6.** Financial toxicity

Why these answers fit, page 158

An assistance application is pending

The support team submits an assistance application. Eligibility and coverage are not yet confirmed. The patient has authorized contact with the program, not the employer.

Complete the exchange using each phrase once. Read both roles, then switch roles and repeat with the bank covered.

WORD AND PHRASE BANK

unconfirmed / employer / submitted / authorized

01 **Patient:** The application has been [1] _____.

02 **Navigator:** Eligibility remains [2] _____.

03 **Patient:** Contact with the program is [3] _____.

04 **Navigator:** That does not authorize contacting your [4] _____.

CHECK THE GAPS

1. submitted / **2.** unconfirmed / **3.** authorized / **4.** employer

Why these answers fit, page 158

Keep the distinction audible

Invite practical concerns without promising coverage or treating cost-related difficulty as refusal.

01 / Answers and explanations

A / Read the situation

A1. What is established?

D. A malignant tumor on pathology

The pathology confirms malignancy; staging and individual outcomes remain unresolved.

Return to the exercise, page 4

A2. What does Maya request?

A. The main finding followed by a pause

Maya's stated preference is a clear main finding and time to respond.

Return to the exercise, page 4

A3. What family decision is unfinished?

C. Whether a relative should join

Maya has not yet chosen whether to include a relative.

Return to the exercise, page 4

D / Response choices

D1. Which opening is clearest?

C. The tissue examination confirms a malignant tumor.

Naming the confirmed tumor states the established finding without obscuring it or inventing later conclusions.

Return to the exercise, page 8

D2. Which response respects Maya's preference?

C. Pause after the main finding.

Maya explicitly requests time to respond before receiving more detail.

Return to the exercise, page 8

01 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: Microscopic study of tissue structure and features. Which term fits?

B. histology

A Not here. patient preference: A person's stated wishes about care or communication. The usual combination is "elicit the patient preference".

B Correct. histology: Microscopic study of tissue structure and features. The usual combination is "review the histology".

C Not here. emotional acknowledgment: Recognition of the listener's expressed feelings or response. The usual combination is "offer emotional acknowledgment".

D Not here. support person: Individual the patient wants involved for assistance. The usual combination is "invite a chosen support person".

Return to the exercise, page 8

D4. Rebuild the exchange from "The first clear sentence". Dr Imani: "I am afraid the result is not what we hoped for. The tissue examination confirms a malignant tumor." Maya: [missing response] Dr Imani: "Of course. We can pause. I can see this is a great deal to take in." Which response belongs in the gap?

C. Please give me a moment. I knew the biopsy might find something, but hearing that is different.

A Not here. This line belongs to turn 10, after Dr Imani says: "Yes. We need those remaining details before completing the treatment discussion or estimating your individual outlook." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 14, after Dr Imani says: "You can decide. We will discuss what you want shared rather than assume that every family member is involved." It addresses that point, not the missing turn in the supplied exchange.

C Correct. This is Maya's response in turn 4. It connects Dr Imani's preceding point with Dr Imani's follow-up. A biopsy provides tissue or cells for examination; here its report has established a malignant finding.

D Not here. This line belongs to turn 6, after Dr Imani says: "Of course. We can pause. I can see this is a great deal to take in." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 8

E / Conversation

1. information preference

Turn 2. Information preference describes Maya's requested amount and sequence of detail for this difficult conversation.

2. malignant tumor

Turn 3. A malignant tumor is the cancerous finding established by the pathology in this fictional case.

01 / Answers and explanations / continued

3. biopsy

Turn 4. A biopsy provides tissue or cells for examination; here its report has established a malignant finding.

4. pathology report

Turn 6. The pathology report contains the interpretation of the examined tissue and supports the established diagnosis.

5. staging workup

Turn 8. The staging workup establishes the extent of disease and is still incomplete in this case.

6. prognostic uncertainty

Turn 10. Prognostic uncertainty means the individual future course cannot be established from the available finding alone.

7. support person

Turn 12. A support person is someone Maya chooses to involve; a relative is not automatically entitled to the information.

8. care navigator

Turn 15. A care navigator helps coordinate access and appointments without replacing the clinician's diagnostic or treatment responsibilities.

9. information overload

Turn 17. Information overload occurs when the volume of detail exceeds what a person can usefully process at the time.

10. second opinion

Turn 19. A second opinion is another qualified assessment and can be discussed without treating the request as disloyalty.

Return to the conversation, page 10

F / Accuracy and scope

F1. Which conclusion exceeds the facts?

B. The diagnosis alone establishes Maya's individual prognosis.

A confirmed diagnosis does not supply the missing stage or an individual outlook.

Return to the exercise, page 12

01 / Answers and explanations / continued

F2. What should precede family involvement?

D. Clarifying Maya's wishes and the appropriate permissions

Family involvement must reflect the patient's wishes and the applicable information-sharing rules.

Return to the exercise, page 12

F / Guided speaking

1. Ren

Alex: I would like Ren to join.

Turn 1. Ren is the chosen partner for the next conversation in this scenario.

2. relatives

Clinician: The permission does not automatically include other relatives.

Turn 2. The specified preference does not authorize broader family disclosure by itself.

3. outstanding

Alex: Some findings remain outstanding.

Turn 3. The next meeting is intended to review findings that are still outstanding.

4. supplied

Clinician: No outcome has been supplied.

Turn 4. The case provides an arrangement for discussion, not a new clinical result.

Return to the conversation, page 13

01 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "The first clear sentence". Dr Imani: "I will answer as honestly as I can, including what we do not know. I will not invent a number." Maya: [missing response] Dr Imani: "You can decide. We will discuss what you want shared rather than assume that every family member is involved." Which response belongs in the gap?

A. Thank you. I might want my brother here, but I have not chosen a support person for this conversation yet.

A Correct. This is Maya's response in turn 12. It connects Dr Imani's preceding point with Dr Imani's follow-up. A support person is someone Maya chooses to involve; a relative is not automatically entitled to the information.

B Not here. This line belongs to turn 20, after Dr Imani says: "We can discuss a second opinion and arrange a follow-up conversation. You do not need every decision settled in this first exchange." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 6, after Dr Imani says: "Of course. We can pause. I can see this is a great deal to take in." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 16, after Dr Imani says: "Our care navigator can help with those arrangements while we keep the clinical questions with the appropriate team." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 13

02 / Answers and explanations

A / Read the situation

A1. Which item is available?

D. A pathology report containing grade

The pathology grade is available while the staging workup is unfinished.

Return to the exercise, page 14

A2. What is pending?

D. Imaging needed for staging

The brief explicitly identifies staging imaging as still pending.

Return to the exercise, page 14

A3. What has Theo confused?

B. Grade with stage

Theo has interpreted the grade as though it were the completed stage.

Return to the exercise, page 14

D / Response choices

D1. Which contrast is correct?

C. Grade concerns microscopic features; stage concerns extent.

The two classifications address different properties and cannot replace each other.

Return to the exercise, page 18

D2. What does pending imaging mean?

A. The required result is not yet available.

Pending describes availability and does not establish the result's clinical meaning.

Return to the exercise, page 18

02 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: Spread of a tumor within its original area. Which term fits?

C. local extent

A Not here. TNM classification: Cancer-specific classification describing tumor, regional nodes, and distant metastasis. The usual combination is "explain the TNM classification".

B Not here. localized disease: Cancer confined to a defined local site in the relevant context. The usual combination is "describe localized disease".

C Correct. local extent: Spread of a tumor within its original area. The usual combination is "assess local extent".

D Not here. patient preference: A person's stated wishes about care or communication. The usual combination is "elicit the patient preference".

Return to the exercise, page 18

D4. Rebuild the exchange from "Two terms on one report". Theo: "I also saw a word about how much the cells resemble ordinary tissue. Where does that fit?" Dr Malik: [missing response] Theo: "What does the staging investigation need to establish beyond what the microscope has shown?" Which response belongs in the gap?

C. Differentiation concerns that resemblance. It can inform grading, but it does not by itself tell us where disease is present.

A Not here. This line belongs to turn 10, after Theo says: "So a nearby lymph node and a finding in a distant organ would not be described identically?" It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 8, after Theo says: "What does the staging investigation need to establish beyond what the microscope has shown?" It addresses that point, not the missing turn in the supplied exchange.

C Correct. This is Dr Malik's response in turn 6. It connects Theo's preceding point with Theo's follow-up. Differentiation concerns how closely tumor cells resemble normal cells and can contribute to grading in the relevant system.

D Not here. This line belongs to turn 4, after Theo says: "Then I may have combined two separate pieces of information when I explained the report at home." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 18

E / Conversation

1. Tumor grade

Turn 2. Tumor grade describes microscopic features under the relevant system and is distinct from the cancer's extent.

02 / Answers and explanations / continued

2. cancer stage

Turn 4. Cancer stage describes extent under the applicable system and remains incomplete while necessary evidence is pending.

3. Differentiation

Turn 6. Differentiation concerns how closely tumor cells resemble normal cells and can contribute to grading in the relevant system.

4. local extent

Turn 8. Local extent describes disease in the original area and is one component rather than the entirety of staging.

5. regional lymph node

Turn 10. A regional lymph node belongs to the relevant nearby drainage region and differs from a distant disease site.

6. indeterminate finding

Turn 12. An indeterminate finding has unresolved nature or significance and is not the same as confirmed cancer spread.

7. Distant metastasis

Turn 14. Distant metastasis means established cancer spread to a distant site, which an uncertain spot does not prove.

8. TNM classification

Turn 16. TNM classifies tumor, regional nodes, and metastasis in cancer-specific ways and is not a universal staging shortcut.

9. prognostic factor

Turn 18. A prognostic factor is associated with the expected course but does not determine one person's exact future.

10. staging completion

Turn 20. Staging completion means the necessary information has been assembled and interpreted rather than assumed from grade alone.

Return to the conversation, page 20

02 / Answers and explanations / continued

F / Accuracy and scope

F1. Which wording preserves an indeterminate finding?

B. Its significance is not yet established.

Indeterminate means uncertainty remains and should not be converted into certainty.

Return to the exercise, page 22

F2. Which claim is unsupported?

B. The grade supplies Theo's exact survival time.

Neither the grade nor the supplied facts establish an individual survival prediction.

Return to the exercise, page 22

F / Guided speaking

1. indeterminate

Patient: The report calls the finding indeterminate.

Turn 1. The supplied report explicitly preserves uncertainty about the finding's nature.

2. reviewing

Clinician: The team is still reviewing it.

Turn 2. Review is the current action, not a completed metastatic diagnosis.

3. established

Patient: Spread has not been established.

Turn 3. The case provides no conclusion that the finding represents cancer spread.

4. confirmed

Clinician: No stage change is confirmed.

Turn 4. A stage change cannot be inferred from the uncertain finding alone.

Return to the conversation, page 23

02 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "Two terms on one report". Theo: "That distinction helps. I would rather the report's uncertainty stay visible than be translated into the worst possibility." Dr Malik: [missing response] Theo: "Will every cancer use the same letters and numbers once those investigations are complete?" Which response belongs in the gap?

C. Exactly. Distant metastasis is a specific conclusion, not a synonym for every finding that needs further review.

A Not here. This line belongs to turn 4, after Theo says: "Then I may have combined two separate pieces of information when I explained the report at home." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 10, after Theo says: "So a nearby lymph node and a finding in a distant organ would not be described identically?" It addresses that point, not the missing turn in the supplied exchange.

C Correct. This is Dr Malik's response in turn 14. It connects Theo's preceding point with Theo's follow-up. Distant metastasis means established cancer spread to a distant site, which an uncertain spot does not prove.

D Not here. This line belongs to turn 8, after Theo says: "What does the staging investigation need to establish beyond what the microscope has shown?" It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 23

03 / Answers and explanations

A / Read the situation

A1. What does the report identify?

D. A potentially relevant tumor alteration

The molecular finding may be relevant, but further interpretation is still required.

Return to the exercise, page 26

A2. What is not established?

A. Suitability or availability of a particular treatment

The case leaves appropriateness and access unresolved rather than promising a treatment.

Return to the exercise, page 26

A3. What kind of testing is described?

A. Tumor testing

The report concerns the tumor and does not establish an inherited finding.

Return to the exercise, page 26

D / Response choices

D1. Which statement preserves the result accurately?

A. The alteration may be relevant; suitability still needs review.

May be relevant preserves the finding without treating unresolved suitability as an established treatment choice.

Return to the exercise, page 30

D2. Which distinction matters for relatives?

A. Tumor findings are not automatically inherited findings.

The type of testing and further assessment determine whether inherited implications are established.

Return to the exercise, page 30

03 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: Genetic change acquired in body cells rather than inherited through germ cells. Which term fits?

C. somatic variant

A Not here. patient preference: A person's stated wishes about care or communication. The usual combination is "elicit the patient preference".

B Not here. predictive biomarker: Feature associated with likelihood of response to a particular treatment. The usual combination is "interpret a predictive biomarker".

C Correct. somatic variant: Genetic change acquired in body cells rather than inherited through germ cells. The usual combination is "explain a somatic variant".

D Not here. companion diagnostic: Test providing information essential for appropriate use of a corresponding treatment. The usual combination is "verify a companion diagnostic".

Return to the exercise, page 30

D4. Rebuild the exchange from "A possible match is not a promise". Dr Sato: "Yes. We need to review the supporting evidence and whether the finding fits an appropriate option for you." Quinn: [missing response] Dr Sato: "That is a fair request. A possible target does not guarantee a response to the associated treatment." Which response belongs in the gap?

A. I would like you to explain the evidence level. A long report can look more certain than the information really is.

A Correct. This is Quinn's response in turn 6. It connects Dr Sato's preceding point with Dr Sato's follow-up. Evidence level describes the strength of support for a proposed clinical use, which can vary between findings.

B Not here. This line belongs to turn 4, after Dr Sato says: "I understand that hope. The finding may be relevant, but it is the start of another clinical discussion." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 16, after Dr Sato says: "Exactly. We can discuss a genetics referral if appropriate, without declaring an inherited result that we do not have." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 18, after Dr Sato says: "Correct. We will explain the limits of the test as well as the findings it reports." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 30

E / Conversation

1. molecular alteration

Turn 2. A molecular alteration is a change in a molecular feature and does not alone establish an effective treatment.

03 / Answers and explanations / continued

2. actionability

Turn 4. Actionability concerns whether a finding can usefully inform a clinical action in the actual context.

3. evidence level

Turn 6. Evidence level describes the strength of support for a proposed clinical use, which can vary between findings.

4. targeted therapy

Turn 8. Targeted therapy acts on a relevant molecular target but is not a guarantee of effectiveness for every patient.

5. treatment access

Turn 10. Treatment access concerns whether an appropriate option can actually be obtained, separate from a report's potential match.

6. somatic variant

Turn 12. A somatic variant is acquired in body cells and is distinct from a change established as inherited.

7. germline variant

Turn 14. A germline variant concerns inherited genetic material, which has not been established by this tumor report alone.

8. variant of uncertain significance

Turn 16. A variant of uncertain significance has an unresolved clinical meaning and should not be presented as a proven treatment target.

9. testing limitation

Turn 18. A testing limitation restricts what the analysis can detect or establish and belongs in a balanced interpretation.

10. genetic counseling

Turn 20. Genetic counseling provides qualified support for understanding genetic findings and implications without assuming inherited risk has already been proved.

Return to the conversation, page 32

03 / Answers and explanations / continued

F / Accuracy and scope

F1. What does uncertain significance mean?

C. Clinical meaning is not established.

The phrase preserves uncertainty rather than supporting a specific action by itself.

Return to the exercise, page 34

F2. Which additional issue belongs in the discussion?

B. Evidence, clinical context, and treatment access

Those unresolved factors affect whether a possible match becomes a useful clinical option.

Return to the exercise, page 34

F / Guided speaking

1. insufficient

Patient: The sample was insufficient for that analysis.

Turn 1. The report describes a sample limitation, not an absent molecular target.

2. unavailable

Clinician: The target result is unavailable.

Turn 2. The analysis did not produce an interpretable result for that target.

3. negative

Patient: That is not the same as a negative result.

Turn 3. Failure to obtain a result cannot be converted into a negative finding.

4. review

Clinician: Further testing needs clinical review.

Turn 4. The next testing decision requires assessment rather than automatic repetition.

Return to the conversation, page 35

03 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "A possible match is not a promise". Dr Sato: "It is not the same category. This is tumor testing; it does not automatically establish an inherited finding." Quinn: [missing response] Dr Sato: "Exactly. We can discuss a genetics referral if appropriate, without declaring an inherited result that we do not have." Which response belongs in the gap?

C. So a germline variant would require the appropriate separate interpretation and, where indicated, further assessment.

A Not here. This line belongs to turn 8, after Dr Sato says: "That is a fair request. A possible target does not guarantee a response to the associated treatment." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 12, after Dr Sato says: "We will. You also asked whether this result means your children have the same genetic change." It addresses that point, not the missing turn in the supplied exchange.

C Correct. This is Quinn's response in turn 14. It connects Dr Sato's preceding point with Dr Sato's follow-up. A germline variant concerns inherited genetic material, which has not been established by this tumor report alone.

D Not here. This line belongs to turn 2, after Dr Sato says: "The tumor report is available. What have you understood from it so far, and what would you like clarified?" It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 35

04 / Answers and explanations

A / Read the situation

A1. What is the proposed aim?

C. Disease control

The case names disease control and explicitly excludes a promised cure.

Return to the exercise, page 36

A2. What matters particularly to Jordan?

B. Caring for a partner

Jordan's caregiving role is the stated personal priority to explore.

Return to the exercise, page 36

A3. What remains undecided?

A. The final treatment choice

The discussion is ongoing and no final treatment decision is supplied.

Return to the exercise, page 36

D / Response choices

D1. Which statement accurately names the goal?

D. This option aims at disease control.

Disease control is the stated aim; no response or cure is promised.

Return to the exercise, page 40

D2. Which question elicits a relevant priority?

A. How would the treatment demands affect caring for your partner?

Asking about the effect on caregiving links treatment demands with Jordan's stated priority.

Return to the exercise, page 40

04 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: Treatment given before the main local treatment in an applicable plan. Which term fits?

A. neoadjuvant treatment

A Correct. neoadjuvant treatment: Treatment given before the main local treatment in an applicable plan. The usual combination is "explain neoadjuvant treatment".

B Not here. patient preference: A person's stated wishes about care or communication. The usual combination is "elicit the patient preference".

C Not here. treatment cycle: Repeated unit of a planned treatment schedule. The usual combination is "explain a treatment cycle".

D Not here. supportive treatment: Intervention addressing symptoms or treatment-related difficulties. The usual combination is "coordinate supportive treatment".

Return to the exercise, page 40

D4. Rebuild the exchange from "What is this option trying to achieve?". Jordan: "I care for my partner. Repeated hospital visits could affect both of us, not only me." Dr Bell: [missing response] Jordan: "I do not want practical difficulties to sound as though I am refusing treatment without listening." Which response belongs in the gap?

D. Your caregiving responsibility belongs in this conversation. Let us examine those demands alongside the clinical aims.

A Not here. This line belongs to turn 16, after Jordan says: "Being able to function at home matters to me. A scan is not the only thing I will judge by." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 14, after Jordan says: "I would also like to understand the other reasonable options, not just the one named first." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 2, after Jordan says: "Before we go through appointments, I need to understand what you hope this treatment will do." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Dr Bell's response in turn 6. It connects Jordan's preceding point with Jordan's follow-up. Caregiving responsibility is a real practical priority that should inform the discussion of treatment demands.

Return to the exercise, page 40

E / Conversation

1. treatment intent

Turn 2. Treatment intent identifies the goal pursued by an option rather than an outcome that has already occurred.

04 / Answers and explanations / continued

2. Disease control

Turn 4. Disease control means limiting growth or spread and does not mean a guaranteed permanent elimination of cancer.

3. caregiving responsibility

Turn 6. Caregiving responsibility is a real practical priority that should inform the discussion of treatment demands.

4. Treatment burden

Turn 8. Treatment burden includes physical, practical, and emotional demands and is relevant to an informed treatment choice.

5. treatment regimen

Turn 10. A treatment regimen is the planned combination and schedule, which must be explained for the actual option rather than invented here.

6. treatment cycle

Turn 12. A treatment cycle is a repeating unit of a planned schedule, not a guarantee of response or a universal length.

7. alternative option

Turn 14. An alternative option is another reasonable approach that belongs in the individualized discussion before a voluntary decision.

8. quality of life

Turn 16. Quality of life concerns experienced well-being and functioning, not merely a radiological measure.

9. Informed consent

Turn 18. Informed consent requires a voluntary decision after appropriate explanation and questions, not merely a convenient appointment or signature.

10. benefit-risk balance

Turn 20. The benefit-risk balance weighs potential benefit against possible harm without treating either as certain.

Return to the conversation, page 42

04 / Answers and explanations / continued

F / Accuracy and scope

F1. Which account of consent is incomplete?

D. A signature alone proves understanding.

Signing a form cannot replace adequate explanation and attention to understanding and choice.

Return to the exercise, page 44

F2. Which distinction is correct?

A. Curative intent is an aim, not a guarantee.

Intent describes the goal pursued and does not establish that it will be achieved.

Return to the exercise, page 44

F / Guided speaking

1. relief

Clinician: The stated aim is symptom relief.

Turn 1. Symptom relief is the goal supplied for this fictional option.

2. agreed

Reese: The schedule is not yet agreed.

Turn 2. The case explicitly leaves the treatment schedule unsettled.

3. demands

Clinician: We need to explain the practical demands.

Turn 3. Reese requests information about the option's practical effect before deciding.

4. open

Reese: My decision remains open.

Turn 4. No final choice is supplied in this ongoing discussion.

Return to the conversation, page 45

04 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "What is this option trying to achieve?". Jordan: "I would also like to understand the other reasonable options, not just the one named first." Dr Bell: [missing response] Jordan: "Being able to function at home matters to me. A scan is not the only thing I will judge by." Which response belongs in the gap?

A. We should compare each alternative option, including the expected benefits, limitations, and demands relevant to you.

A Correct. This is Dr Bell's response in turn 14. It connects Jordan's preceding point with Jordan's follow-up. An alternative option is another reasonable approach that belongs in the individualized discussion before a voluntary decision.

B Not here. This line belongs to turn 2, after Jordan says: "Before we go through appointments, I need to understand what you hope this treatment will do." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 6, after Jordan says: "I care for my partner. Repeated hospital visits could affect both of us, not only me." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 16, after Jordan says: "Being able to function at home matters to me. A scan is not the only thing I will judge by." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 45

05 / Answers and explanations

A / Read the situation

A1. What is reported?

C. Fever and shaking after recent treatment

Lee reports symptoms; the cause has not been established.

Return to the exercise, page 48

A2. What action is already taken?

B. The urgent assessment pathway is activated

The brief says the service's urgent pathway has already been activated.

Return to the exercise, page 48

A3. What remains unknown?

C. Cause and laboratory results

The handoff must preserve these unknowns rather than manufacture findings.

Return to the exercise, page 48

D / Response choices

D1. Which opening best supports the handoff?

D. Urgent symptom report after recent treatment; the pathway is activated.

Naming the urgent report and activated pathway communicates the concern and action without inventing a diagnosis.

Return to the exercise, page 52

D2. Which diagnosis cannot be asserted from this brief?

A. Confirmed neutropenia

A low neutrophil count has not been established because laboratory results are unknown.

Return to the exercise, page 52

05 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: Measurement of the number of neutrophils in blood. Which term fits?

C. neutrophil count

A Not here. patient preference: A person's stated wishes about care or communication. The usual combination is "elicit the patient preference".

B Not here. urgent assessment: Prompt clinical evaluation through the appropriate service pathway. The usual combination is "arrange urgent assessment".

C Correct. neutrophil count: Measurement of the number of neutrophils in blood. The usual combination is "review the neutrophil count".

D Not here. mucositis: Inflammation affecting mucous membranes. The usual combination is "describe mucositis symptoms".

Return to the exercise, page 52

D4. Rebuild the exchange from "The call cannot become a routine message". Ana: "I will also verify the most recent treatment information from the actual record rather than rely on memory." Dr Ford: [missing response] Ana: "Shall I describe this as confirmed treatment toxicity in the handoff heading, or keep the cause open?" Which response belongs in the gap?

B. Yes. The last treatment date matters, but do not delay the active response while completing every detail.

A Not here. This line belongs to turn 8, after Ana says: "Shall I describe this as confirmed treatment toxicity in the handoff heading, or keep the cause open?" It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Dr Ford's response in turn 6. It connects Ana's preceding point with Ana's follow-up. The last treatment date provides relevant clinical context and should be verified from the appropriate source.

C Not here. This line belongs to turn 14, after Ana says: "I need a clear receiving contact so that the report does not move between teams without an owner." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 10, after Ana says: "I have not labeled Lee neutropenic. There is no count available to support that description yet." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 52

E / Conversation

1. urgent assessment

Turn 2. Urgent assessment is the activated clinical pathway in this case, not a routine message to be reviewed later.

05 / Answers and explanations / continued

2. symptom onset time

Turn 4. Symptom onset time identifies when the symptoms began and must be confirmed rather than guessed during the handoff.

3. last treatment date

Turn 6. The last treatment date provides relevant clinical context and should be verified from the appropriate source.

4. causality assessment

Turn 8. Causality assessment determines whether treatment caused an event; that conclusion is not supplied by timing alone.

5. neutrophil count

Turn 10. A neutrophil count is a laboratory measurement that has not yet been supplied in this scenario.

6. infection concern

Turn 12. An infection concern is a clinical suspicion requiring assessment, not a confirmed infection diagnosis.

7. receiving clinician

Turn 14. The receiving clinician is the professional accepting the next clinical assessment and responsibility in the handoff.

8. read-back

Turn 16. Read-back repeats important details to confirm accuracy rather than assuming the message was understood correctly.

9. escalation acknowledgment

Turn 18. Escalation acknowledgment records that the urgent request was received and accepted, beyond merely attempting contact.

10. adverse event

Turn 20. An adverse event is an unfavorable occurrence during care and does not by itself prove treatment causation.

Return to the conversation, page 54

05 / Answers and explanations / continued

F / Accuracy and scope

F1. What must the handoff confirm?

D. The receiving clinician and responsibility

A handoff requires an accepted next step rather than relying on a message's existence.

Return to the exercise, page 56

F2. Which statement preserves causality correctly?

D. Symptoms occurred after treatment; the cause is not established.

Temporal sequence does not prove causality and does not remove the need for urgent assessment.

Return to the exercise, page 56

F / Guided speaking

1. active

Nurse: The urgent pathway is active.

Turn 1. The scenario begins with the actual urgent response already in progress.

2. accept

Dr Lane: I accept the handoff.

Turn 2. Dr Lane explicitly accepts responsibility for receiving the clinical handoff.

3. pending

Nurse: The laboratory results remain pending.

Turn 3. The results have not yet become available in the supplied facts.

4. established

Dr Lane: The cause is not established.

Turn 4. Neither the symptom report nor the handoff establishes a cause by itself.

Return to the conversation, page 57

05 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "The call cannot become a routine message". Ana: "I need a clear receiving contact so that the report does not move between teams without an owner." Dr Ford: [missing response] Ana: "I will repeat the patient's reported symptoms and the actions already taken, then confirm the next communication." Which response belongs in the gap?

C. The receiving clinician must be identified through our pathway. I am accepting this clinical discussion now.

A Not here. This line belongs to turn 10, after Ana says: "I have not labeled Lee neutropenic. There is no count available to support that description yet." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 8, after Ana says: "Shall I describe this as confirmed treatment toxicity in the handoff heading, or keep the cause open?" It addresses that point, not the missing turn in the supplied exchange.

C Correct. This is Dr Ford's response in turn 14. It connects Ana's preceding point with Ana's follow-up. The receiving clinician is the professional accepting the next clinical assessment and responsibility in the handoff.

D Not here. This line belongs to turn 18, after Ana says: "I will document that the concern was received and who accepted it, not simply that a call was attempted." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 57

06 / Answers and explanations

A / Read the situation

A1. What does the report describe?

C. A measured lesion is smaller

Only the specific size comparison is supplied, not a complete response conclusion.

Return to the exercise, page 58

A2. What is still under review?

D. The complete response assessment

The complete assessment has not yet been finalized by oncology.

Return to the exercise, page 58

A3. What has not been agreed?

C. A treatment change

The brief supplies no agreed change to the treatment plan.

Return to the exercise, page 58

D / Response choices

D1. Which update is supported?

A. One measured lesion is smaller; the full assessment remains under review.

The supported statement keeps the favorable measurement separate from the unfinished overall interpretation.

Return to the exercise, page 63

D2. Which claim confuses observation and classification?

D. Smaller automatically means formal partial response.

Formal response categories require their applicable criteria and the full relevant assessment.

Return to the exercise, page 63

06 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: Selected lesion followed for measurement under a response-assessment system. Which term fits?

A. target lesion

A Correct. target lesion: Selected lesion followed for measurement under a response-assessment system. The usual combination is "measure a target lesion".

B Not here. progressive disease: Worsening that meets applicable progression criteria. The usual combination is "evaluate progressive disease".

C Not here. measurement variability: Differences arising from how measurements are obtained or interpreted. The usual combination is "account for measurement variability".

D Not here. patient preference: A person's stated wishes about care or communication. The usual combination is "elicit the patient preference".

Return to the exercise, page 63

D4. Rebuild the exchange from "Smaller is useful, but not the whole conclusion". Pat: "Does smaller automatically mean the treatment has produced what the team calls a partial response?" Dr Ng: [missing response] Pat: "I noticed the report described other findings as well. Are they part of the same review?" Which response belongs in the gap?

D. A partial response has defined criteria. I need the full assessment before using that label for your results.

A Not here. This line belongs to turn 18, after Pat says: "Does the smaller measurement mean you have already decided to change the treatment schedule?" It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 2, after Pat says: "The report says the spot is smaller. I have been waiting to hear whether I can call that good news." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 4, after Pat says: "I would like to understand the finding without saying more than the report can actually support." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Dr Ng's response in turn 6. It connects Pat's preceding point with Pat's follow-up. A partial response must meet the applicable assessment criteria and cannot be inferred from the word smaller alone.

Return to the exercise, page 63

E / Conversation

1. comparison scan

Turn 2. A comparison scan provides the earlier imaging reference for the reported change in the measured lesion.

06 / Answers and explanations / continued

2. response assessment

Turn 4. Response assessment considers the relevant evidence and criteria rather than assigning a category from one descriptive word.

3. partial response

Turn 6. A partial response must meet the applicable assessment criteria and cannot be inferred from the word smaller alone.

4. target lesion

Turn 8. A target lesion is selected for measurement under an assessment system and does not represent every relevant disease finding.

5. Non-target lesions

Turn 10. Non-target lesions are relevant disease findings assessed outside the selected target measurements and may affect interpretation.

6. Clinical response

Turn 12. Clinical response concerns changes in symptoms, signs, or functioning and adds information distinct from imaging measurements.

7. complete response

Turn 14. Complete response refers to disappearance of detectable findings under applicable criteria and has not been established here.

8. Remission

Turn 16. Remission describes reduction or disappearance of disease signs and should not be used as an automatic guarantee of permanent cure.

9. assessment criteria

Turn 18. Assessment criteria are the defined rules used to classify changes and support interpretation rather than an automatic schedule change.

10. radiological response

Turn 20. Radiological response is change assessed on imaging and will be discussed with the full clinical picture when the review is complete.

Return to the conversation, page 65

06 / Answers and explanations / continued

F / Accuracy and scope

F1. Which treatment statement is accurate?

A. No change has been agreed.

The case explicitly leaves any treatment change undecided.

Return to the exercise, page 67

F2. Why avoid equating complete response with cure?

B. They describe different claims about disease and future outcome.

A response category does not automatically establish a permanent individual outcome.

Return to the exercise, page 67

F / Guided speaking

1. new

Patient: The report describes a new finding.

Turn 1. New describes the observation supplied, not its established significance.

2. incomplete

Clinician: The review remains incomplete.

Turn 2. The oncology interpretation has not yet been completed in this scenario.

3. classified

Patient: Progression is not yet classified.

Turn 3. The case does not supply a formal progression category.

4. agreed

Clinician: No treatment change is agreed.

Turn 4. A new observation does not by itself establish an agreed treatment change.

Return to the conversation, page 68

06 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "Smaller is useful, but not the whole conclusion". Pat: "I want to tell my family there is an encouraging change, but not that everything has disappeared." Dr Ng: [missing response] Pat: "And even that term would need explaining rather than being used as a simple synonym for cure?" Which response belongs in the gap?

B. That is accurate. A complete response has not been established by the facts we are discussing.

A Not here. This line belongs to turn 2, after Pat says: "The report says the spot is smaller. I have been waiting to hear whether I can call that good news." It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Dr Ng's response in turn 14. It connects Pat's preceding point with Pat's follow-up. Complete response refers to disappearance of detectable findings under applicable criteria and has not been established here.

C Not here. This line belongs to turn 8, after Pat says: "I noticed the report described other findings as well. Are they part of the same review?" It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 20, after Pat says: "Then my summary is that one lesion is smaller, the full review is pending, and the plan has not changed yet." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 68

07 / Answers and explanations

A / Read the situation

A1. What is the participation status?

D. Eligibility review incomplete; no consent or enrollment

The scenario keeps eligibility, consent, and enrollment as unfinished and distinct stages.

Return to the exercise, page 71

A2. How does the trial assign approaches?

B. Random assignment

The brief explicitly states random assignment without describing a guaranteed superior group.

Return to the exercise, page 71

A3. What worries Val?

B. Declining might harm the care relationship

Val's concern is pressure on the relationship rather than a supplied clinical result.

Return to the exercise, page 71

D / Response choices

D1. Which statement separates the stages correctly?

D. Eligibility is under review; Val is not enrolled.

The case gives neither completed eligibility nor consent nor enrollment.

Return to the exercise, page 75

D2. Which claim creates therapeutic misconception?

C. The research exists solely to give Val the best individual treatment.

Research evaluates a question and cannot be presented as a guarantee of individualized benefit.

Return to the exercise, page 75

07 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: Formal entry into a study after required processes. Which term fits?

B. enrollment

A Not here. standard of care: Accepted clinical approach for the relevant setting and circumstances. The usual combination is "compare with standard of care".

B Correct. enrollment: Formal entry into a study after required processes. The usual combination is "distinguish enrollment from screening".

C Not here. patient preference: A person's stated wishes about care or communication. The usual combination is "elicit the patient preference".

D Not here. withdrawal: Ending participation through the applicable study process. The usual combination is "explain withdrawal options".

Return to the exercise, page 75

D4. Rebuild the exchange from "An option to discuss, not an obligation". Val: "I would like to understand what the study is trying to find out before considering any forms." Dr Ortiz: [missing response] Val: "Would you choose whichever treatment group you think is best for me after I join?" Which response belongs in the gap?

C. The research protocol describes its question, procedures, and participant requirements. We will review the relevant details with you.

A Not here. This line belongs to turn 10, after Val says: "That matters to my decision. I do not want to assume that the new option is already known to be better." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 2, after Val says: "You mentioned a trial. I worry that saying no would make you think I am not cooperating with care." It addresses that point, not the missing turn in the supplied exchange.

C Correct. This is Dr Ortiz's response in turn 6. It connects Val's preceding point with Val's follow-up. The research protocol sets out the study's purpose and procedures and should guide an accurate explanation of participation.

D Not here. This line belongs to turn 4, after Val says: "Am I already accepted because you brought it up, or is there another process before that?" It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 75

E / Conversation

1. clinical trial

Turn 2. A clinical trial is research evaluating an intervention, and an invitation to discuss it is not an obligation to join.

07 / Answers and explanations / continued

2. eligibility criteria

Turn 4. Eligibility criteria are entry requirements that remain under review and do not establish enrollment by themselves.

3. research protocol

Turn 6. The research protocol sets out the study's purpose and procedures and should guide an accurate explanation of participation.

4. randomization

Turn 8. Randomization assigns study groups through a chance-based process rather than the clinician selecting an individually preferred arm.

5. investigational treatment

Turn 10. An investigational treatment is being studied for the relevant use and is not proven superior merely because it is new.

6. standard of care

Turn 12. Standard of care describes accepted clinical approaches in context and belongs in the discussion of alternatives to research participation.

7. research coordinator

Turn 14. A research coordinator can clarify study arrangements and visits while clinical and consent questions remain with the appropriate professionals.

8. withdrawal

Turn 16. Withdrawal is ending participation through the applicable process, with its details and continuing care discussed accurately.

9. consent discussion

Turn 18. A consent discussion supports understanding and a voluntary decision rather than treating a signature as the whole process.

10. Enrollment

Turn 20. Enrollment is formal entry after the required study processes and has not occurred in this scenario.

Return to the conversation, page 77

07 / Answers and explanations / continued

F / Accuracy and scope

F1. What does randomization mean?

B. Chance-based assignment under the study process

Randomization assigns groups through chance rather than individualized selection of a supposedly superior group.

Return to the exercise, page 79

F2. Which response to Val's concern is appropriate?

B. Declining research does not remove the right to appropriate care.

Voluntary participation requires avoiding coercion and clarifying continuing care.

Return to the exercise, page 79

F / Guided speaking

1. screening

Jo: I agreed to a screening visit.

Turn 1. Jo agreed to an eligibility-related visit, not completed study enrollment.

2. unknown

Clinician: Your eligibility remains unknown.

Turn 2. The entry requirements have not yet been established as met.

3. enrolled

Jo: I have not enrolled in the study.

Turn 3. The case explicitly states that formal study entry has not occurred.

4. withdrawal

Clinician: We will explain the withdrawal process.

Turn 4. Jo asks about ending participation before deciding whether to participate.

Return to the conversation, page 80

07 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "An option to discuss, not an obligation". Val: "The visits and tests may affect my work. Can I ask about those before deciding whether to proceed?" Dr Ortiz: [missing response] Val: "If I first agree and later want to stop, I need to understand what happens to my care." Which response belongs in the gap?

D. Certainly. The research coordinator can help explain practical arrangements, alongside our discussion of the clinical questions.

A Not here. This line belongs to turn 8, after Val says: "Would you choose whichever treatment group you think is best for me after I join?" It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 20, after Val says: "Then eligibility is still being checked, and I have not consented or joined by having this conversation." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 16, after Val says: "If I first agree and later want to stop, I need to understand what happens to my care." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Dr Ortiz's response in turn 14. It connects Val's preceding point with Val's follow-up. A research coordinator can clarify study arrangements and visits while clinical and consent questions remain with the appropriate professionals.

Return to the exercise, page 80

08 / Answers and explanations

A / Read the situation

A1. Why is the consultation offered?

B. Symptoms and distress

The referral addresses symptoms and distress while the treatment discussion continues.

Return to the exercise, page 81

A2. What does Rowan fear?

B. That treatment is being stopped

Rowan interprets palliative care as a decision to end anticancer treatment.

Return to the exercise, page 81

A3. Which preference is stated?

A. More comfortable time with family

Rowan names comfort and family time, not a completed decision about other care.

Return to the exercise, page 81

D / Response choices

D1. Which statement explains the referral correctly?

A. Palliative care can support symptoms alongside treatment.

Palliative support can accompany other treatment and does not settle separate care decisions.

Return to the exercise, page 85

D2. Which question explores the misunderstanding?

D. What did the phrase suggest to you?

Asking what the phrase suggested invites Rowan's interpretation without blame or a leading conclusion.

Return to the exercise, page 85

08 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: Conversation about priorities and how care should respond to them. Which term fits?

C. goals-of-care discussion

A Not here. spiritual concern: Concern about meaning, belief, connection, or existential distress. The usual combination is "invite a spiritual concern".

B Not here. patient preference: A person's stated wishes about care or communication. The usual combination is "elicit the patient preference".

C Correct. goals-of-care discussion: Conversation about priorities and how care should respond to them. The usual combination is "open a goals-of-care discussion".

D Not here. resuscitation status: Documented decision or order about resuscitation in the relevant setting. The usual combination is "clarify resuscitation status".

Return to the exercise, page 85

D4. Rebuild the exchange from "Support alongside treatment". Rowan: "Then please tell me what the additional team would actually help with in everyday life." Dr Ahmed: [missing response] Rowan: "I want more comfortable time with my family. At present the symptoms take over the entire day." Which response belongs in the gap?

A. They can help with symptom management, distress, and the effects of serious illness on you and those supporting you.

A Correct. This is Dr Ahmed's response in turn 6. It connects Rowan's preceding point with Rowan's follow-up. Symptom management addresses distressing symptoms and is one reason for this consultation while other treatment discussions continue.

B Not here. This line belongs to turn 2, after Rowan says: "When you mentioned palliative care, I heard that you were giving up on the cancer treatment." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 16, after Rowan says: "Could we have a conversation together where she can hear what I want without taking over the discussion?" It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 14, after Rowan says: "My daughter helps a great deal, but I do not want everyone to assume she makes all decisions for me." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 85

E / Conversation

1. palliative care

Turn 2. Palliative care addresses symptoms and quality of life in serious illness and can accompany anticancer treatment.

08 / Answers and explanations / continued

2. concurrent care

Turn 4. Concurrent care means approaches can be provided alongside one another, rather than assuming one automatically replaces another.

3. symptom management

Turn 6. Symptom management addresses distressing symptoms and is one reason for this consultation while other treatment discussions continue.

4. symptom burden

Turn 8. Symptom burden is the combined effect of symptoms on daily life and helps make Rowan's stated priority concrete.

5. Hospice care

Turn 10. Hospice care has a distinct comfort-focused purpose and applicable service criteria; it is not automatically established by this referral.

6. resuscitation status

Turn 12. Resuscitation status is a separate documented care decision and is not determined by accepting a palliative-care consultation.

7. surrogate decision-maker

Turn 14. A surrogate decision-maker acts under applicable authority when required, which is not established merely by being a helpful relative.

8. family meeting

Turn 16. A family meeting is a planned conversation with the appropriate people, guided by the patient's wishes and information-sharing arrangements.

9. Caregiver strain

Turn 18. Caregiver strain describes the burden on someone providing care and can be addressed without replacing the patient's own voice.

10. goals-of-care discussion

Turn 20. A goals-of-care discussion explores priorities and appropriate care rather than automatically ending treatment or assigning a resuscitation status.

Return to the conversation, page 87

08 / Answers and explanations / continued

F / Accuracy and scope

F1. What priority should remain visible?

A. Comfortable time with family

Comfortable family time is Rowan's explicit priority in this fictional conversation.

Return to the exercise, page 89

F2. Which assumption is unsafe?

D. Every relative present has automatic decision-making authority.

Presence or relationship alone does not establish authority under the relevant legal arrangements.

Return to the exercise, page 89

F / Guided speaking

1. symptoms

Taylor: I want help with my symptoms.

Turn 1. Symptom support is the care need Taylor explicitly identifies.

2. review

Clinician: Treatment options remain under review.

Turn 2. The options are still being reviewed and no treatment decision is supplied.

3. Jo

Taylor: I would like Jo at the meeting.

Turn 3. Jo is the friend Taylor wishes to include in the discussion.

4. authority

Clinician: Attendance does not establish decision-making authority.

Turn 4. Being invited to a meeting does not automatically create legal decision-making authority.

Return to the conversation, page 90

08 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "Support alongside treatment". Rowan: "My daughter helps a great deal, but I do not want everyone to assume she makes all decisions for me." Dr Ahmed: [missing response] Rowan: "Could we have a conversation together where she can hear what I want without taking over the discussion?" Which response belongs in the gap?

B. We should clarify that. A surrogate decision-maker has a specific role under the applicable arrangements, not simply because a relative attends.

A Not here. This line belongs to turn 2, after Rowan says: "When you mentioned palliative care, I heard that you were giving up on the cancer treatment." It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Dr Ahmed's response in turn 14. It connects Rowan's preceding point with Rowan's follow-up. A surrogate decision-maker acts under applicable authority when required, which is not established merely by being a helpful relative.

C Not here. This line belongs to turn 6, after Rowan says: "Then please tell me what the additional team would actually help with in everyday life." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 4, after Rowan says: "I thought it meant every other treatment would stop, and that I would no longer see this team." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 90

Review 1 / Explanations

Cumulative review

1. A colleague needs the term for this meaning: Cancerous disease characterized by malignant cells or tissue. Which term fits?

C. malignancy

A Not here. care navigator: Professional helping a person move through services and appointments. The usual combination is "contact the care navigator".

B Not here. teach-back: Checking an explanation by inviting the listener to restate its meaning. The usual combination is "use teach-back without blame".

C Correct. malignancy: Cancerous disease characterized by malignant cells or tissue. The usual combination is "explain a confirmed malignancy".

D Not here. information preference: How much detail a person wants and how they want it presented. The usual combination is "check the information preference".

Return to the exercise, page 24

2. A colleague needs the term for this meaning: Stage incorporating surgical and pathological findings where applicable. Which term fits?

A. pathological stage

A Correct. pathological stage: Stage incorporating surgical and pathological findings where applicable. The usual combination is "distinguish pathological stage".

B Not here. indeterminate finding: Finding whose nature or significance is not yet established. The usual combination is "explain an indeterminate finding".

C Not here. tumor grade: Classification of microscopic tumor features under the relevant grading system. The usual combination is "explain tumor grade".

D Not here. qualified interpreter: Trained language professional supporting accurate communication between speakers. The usual combination is "request a qualified interpreter".

Return to the exercise, page 24

Review 1 / Explanations / continued

3. Rebuild the exchange from "The first clear sentence". Dr Imani: "It establishes the cancer diagnosis. It does not yet give us every answer about the extent or treatment." Maya: [missing response] Dr Imani: "Yes. We need those remaining details before completing the treatment discussion or estimating your individual outlook." Which response belongs in the gap?

B. So the staging workup is a separate part of understanding what happens next?

A Not here. This line belongs to turn 2, after Dr Imani says: "Before we discuss the report, how much detail would you like in this first conversation?" It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Maya's response in turn 8. It connects Dr Imani's preceding point with Dr Imani's follow-up. The staging workup establishes the extent of disease and is still incomplete in this case.

C Not here. This line belongs to turn 14, after Dr Imani says: "You can decide. We will discuss what you want shared rather than assume that every family member is involved." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 18, after Dr Imani says: "That would risk information overload. We will give manageable information and a clear contact route." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 24

4. Rebuild the exchange from "Two terms on one report". Theo: "And even when the stage is known, it will not tell you exactly what happens to me personally?" Dr Malik: [missing response] Theo: "Then I can say the grade is reported, but the stage still needs the remaining investigation results." Which response belongs in the gap?

B. Correct. A prognostic factor informs a discussion; it is not an exact individual timetable.

A Not here. This line belongs to turn 20, after Theo says: "Then I can say the grade is reported, but the stage still needs the remaining investigation results." It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Dr Malik's response in turn 18. It connects Theo's preceding point with Theo's follow-up. A prognostic factor is associated with the expected course but does not determine one person's exact future.

C Not here. This line belongs to turn 16, after Theo says: "Will every cancer use the same letters and numbers once those investigations are complete?" It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 2, after Theo says: "My report gives a grade. I thought that meant you already knew the stage of the cancer." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 24

Review 2 / Explanations

Cumulative review

1. A colleague needs the term for this meaning: Treatment directed at a relevant molecular target involved in disease. Which term fits?

C. targeted therapy

A Not here. evidence level: Strength or category of support for a clinical claim or action. The usual combination is "review the evidence level".

B Not here. acquired resistance: Reduced treatment sensitivity developing during or after exposure. The usual combination is "investigate acquired resistance".

C Correct. targeted therapy: Treatment directed at a relevant molecular target involved in disease. The usual combination is "evaluate a targeted therapy".

D Not here. teach-back: Checking an explanation by inviting the listener to restate its meaning. The usual combination is "use teach-back without blame".

Return to the exercise, page 46

2. A colleague needs the term for this meaning: Planned combination and schedule of treatment components. Which term fits?

C. treatment regimen

A Not here. treatment intent: Goal a treatment is intended to pursue. The usual combination is "clarify treatment intent".

B Not here. qualified interpreter: Trained language professional supporting accurate communication between speakers. The usual combination is "request a qualified interpreter".

C Correct. treatment regimen: Planned combination and schedule of treatment components. The usual combination is "review the treatment regimen".

D Not here. benefit-risk balance: Weighing expected benefits against possible harms. The usual combination is "explain the benefit-risk balance".

Return to the exercise, page 46

Review 2 / Explanations / continued

3. Rebuild the exchange from "Two terms on one report". Theo: "So a nearby lymph node and a finding in a distant organ would not be described identically?" Dr Malik: [missing response] Theo: "I have heard people use the word metastatic for any unusual spot. Is that too definite?" Which response belongs in the gap?

A. Correct. A regional lymph node and a distant site have different roles in the applicable staging system.

A Correct. This is Dr Malik's response in turn 10. It connects Theo's preceding point with Theo's follow-up. A regional lymph node belongs to the relevant nearby drainage region and differs from a distant disease site.

B Not here. This line belongs to turn 4, after Theo says: "Then I may have combined two separate pieces of information when I explained the report at home." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 8, after Theo says: "What does the staging investigation need to establish beyond what the microscope has shown?" It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 12, after Theo says: "I have heard people use the word metastatic for any unusual spot. Is that too definite?" It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 46

4. Rebuild the exchange from "What is this option trying to achieve?". Jordan: "I am not ready to sign anything just because an appointment slot might be available." Dr Bell: [missing response] Jordan: "Then let us compare the possible benefit with the burdens before calling this a settled plan." Which response belongs in the gap?

D. Informed consent is a discussion and a voluntary decision, not simply obtaining a signature.

A Not here. This line belongs to turn 10, after Jordan says: "Could you explain the proposed schedule, then distinguish what is definite from what may change?" It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 8, after Jordan says: "I do not want practical difficulties to sound as though I am refusing treatment without listening." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 2, after Jordan says: "Before we go through appointments, I need to understand what you hope this treatment will do." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Dr Bell's response in turn 18. It connects Jordan's preceding point with Jordan's follow-up. Informed consent requires a voluntary decision after appropriate explanation and questions, not merely a convenient appointment or signature.

Return to the exercise, page 46

Review 3 / Explanations

Cumulative review

1. A colleague needs the term for this meaning: Abnormally low neutrophil count. Which term fits?

C. neutropenia

A Not here. extravasation: Escape of an infused substance from a vessel into surrounding tissue. The usual combination is "escalate suspected extravasation".

B Not here. last treatment date: Most recent date on which treatment was administered. The usual combination is "verify the last treatment date".

C Correct. neutropenia: Abnormally low neutrophil count. The usual combination is "assess for neutropenia".

D Not here. teach-back: Checking an explanation by inviting the listener to restate its meaning. The usual combination is "use teach-back without blame".

Return to the exercise, page 69

2. A colleague needs the term for this meaning: Disease not meeting the relevant response or progression thresholds. Which term fits?

C. stable disease

A Not here. qualified interpreter: Trained language professional supporting accurate communication between speakers. The usual combination is "request a qualified interpreter".

B Not here. remission: Reduction or disappearance of signs of disease, described as partial or complete when appropriate. The usual combination is "explain remission".

C Correct. stable disease: Disease not meeting the relevant response or progression thresholds. The usual combination is "discuss stable disease".

D Not here. response assessment: Evaluation of how disease has changed during or after treatment. The usual combination is "complete a response assessment".

Return to the exercise, page 69

Review 3 / Explanations / continued

3. Rebuild the exchange from "What is this option trying to achieve?". Jordan: "Could you explain the proposed schedule, then distinguish what is definite from what may change?" Dr Bell: [missing response] Jordan: "I have heard cycle used as though everyone knows what it means. Please explain that too." Which response belongs in the gap?

A. We would review the treatment regimen and the circumstances in which it might need review.

A Correct. This is Dr Bell's response in turn 10. It connects Jordan's preceding point with Jordan's follow-up. A treatment regimen is the planned combination and schedule, which must be explained for the actual option rather than invented here.

B Not here. This line belongs to turn 6, after Jordan says: "I care for my partner. Repeated hospital visits could affect both of us, not only me." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 2, after Jordan says: "Before we go through appointments, I need to understand what you hope this treatment will do." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 8, after Jordan says: "I do not want practical difficulties to sound as though I am refusing treatment without listening." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 69

4. Rebuild the exchange from "Smaller is useful, but not the whole conclusion". Pat: "Does the smaller measurement mean you have already decided to change the treatment schedule?" Dr Ng: [missing response] Pat: "Then my summary is that one lesion is smaller, the full review is pending, and the plan has not changed yet." Which response belongs in the gap?

D. No. We must finish the review using the relevant assessment criteria and clinical context before discussing any recommendation.

A Not here. This line belongs to turn 6, after Pat says: "Does smaller automatically mean the treatment has produced what the team calls a partial response?" It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 4, after Pat says: "I would like to understand the finding without saying more than the report can actually support." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 2, after Pat says: "The report says the spot is smaller. I have been waiting to hear whether I can call that good news." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Dr Ng's response in turn 18. It connects Pat's preceding point with Pat's follow-up. Assessment criteria are the defined rules used to classify changes and support interpretation rather than an automatic schedule change.

Return to the exercise, page 69

Review 4 / Explanations

Cumulative review

1. A colleague needs the term for this meaning: Assessment checking whether trial-entry requirements are met. Which term fits?

C. screening visit

A Not here. teach-back: Checking an explanation by inviting the listener to restate its meaning. The usual combination is "use teach-back without blame".

B Not here. voluntary participation: Taking part by choice without improper pressure. The usual combination is "protect voluntary participation".

C Correct. screening visit: Assessment checking whether trial-entry requirements are met. The usual combination is "arrange a screening visit".

D Not here. investigational treatment: Treatment being studied for the relevant use. The usual combination is "discuss an investigational treatment".

Return to the exercise, page 91

2. A colleague needs the term for this meaning: Person authorized to decide when the patient cannot under applicable rules. Which term fits?

A. surrogate decision-maker

A Correct. surrogate decision-maker: Person authorized to decide when the patient cannot under applicable rules. The usual combination is "clarify the surrogate decision-maker".

B Not here. palliative care: Care addressing symptoms, distress, and quality of life in serious illness. The usual combination is "introduce palliative care".

C Not here. family meeting: Planned discussion involving the patient and appropriate family or support people. The usual combination is "arrange a family meeting".

D Not here. qualified interpreter: Trained language professional supporting accurate communication between speakers. The usual combination is "request a qualified interpreter".

Return to the exercise, page 91

Review 4 / Explanations / continued

3. Rebuild the exchange from "Smaller is useful, but not the whole conclusion". Pat: "Then I should not assume that measuring one selected area means every other area has been ignored." Dr Ng: [missing response] Pat: "My symptoms have also changed. Should I describe those even if the scan appears encouraging?" Which response belongs in the gap?

A. Correct. Non-target lesions and any new findings may need assessment under the appropriate system.

A Correct. This is Dr Ng's response in turn 10. It connects Pat's preceding point with Pat's follow-up. Non-target lesions are relevant disease findings assessed outside the selected target measurements and may affect interpretation.

B Not here. This line belongs to turn 12, after Pat says: "My symptoms have also changed. Should I describe those even if the scan appears encouraging?" It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 16, after Pat says: "And even that term would need explaining rather than being used as a simple synonym for cure?" It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 14, after Pat says: "I want to tell my family there is an encouraging change, but not that everything has disappeared." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 91

4. Rebuild the exchange from "Support alongside treatment". Rowan: "I also want someone to notice how exhausted she is. She keeps saying she is fine." Dr Ahmed: [missing response] Rowan: "I understand better now. The referral adds symptom support; it does not silently settle every future decision." Which response belongs in the gap?

A. Caregiver strain is worth discussing, with her involvement and the appropriate support.

A Correct. This is Dr Ahmed's response in turn 18. It connects Rowan's preceding point with Rowan's follow-up. Caregiver strain describes the burden on someone providing care and can be addressed without replacing the patient's own voice.

B Not here. This line belongs to turn 10, after Rowan says: "Does agreeing to this consultation mean I have agreed to hospice without realizing it?" It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 12, after Rowan says: "I also do not want one referral to become a decision about resuscitation without anyone asking me." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 8, after Rowan says: "I want more comfortable time with my family. At present the symptoms take over the entire day." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 91

Additional case 1 / Explanations

Extended conversation

1. pathology report

Turn 4. The pathology report contains the relevant tissue-assessment findings and is more specific than an abbreviated diagnosis in an appointment letter.

2. imaging studies

Turn 6. Imaging studies include the acquired examinations used for review and are distinct from the written summaries describing them.

3. treatment summary

Turn 8. A treatment summary records relevant prior therapy and must be checked across sites rather than assumed complete from one institution's list.

4. record-transfer authorization

Turn 12. Record-transfer authorization defines the appropriate permission for sending specified information to the receiving service through the approved process.

5. referral packet

Turn 16. A referral packet is the collection of relevant documents and materials required by the receiving service and may need more than one uploaded file.

6. Appointment confirmation

Turn 20. Appointment confirmation establishes an actual scheduled review and is separate from a request being sent or records being received.

Return to the conversation, page 94

Transfer exchange

1. missing

Patient: The imaging studies are still missing.

Turn 1. The receiving service has not yet received the actual imaging studies.

Additional case 1 / Explanations / continued

2. arrived

Clinician: The report and summary have arrived.

Turn 2. The scenario confirms receipt of those two documents only.

3. complete

Patient: The referral packet is not complete.

Turn 3. Missing requested imaging means the packet remains incomplete.

4. confirmed

Clinician: The appointment is not yet confirmed.

Turn 4. No scheduled review appointment has been confirmed in this update.

Return to the conversation, page 96

Additional case 2 / Explanations

Extended conversation

1. discharge summary

Turn 3. The discharge summary records the hospital episode and discharge instructions but must be checked against the actual current plan and medicine information.

2. medication discrepancy

Turn 6. A medication discrepancy is a conflict between medicine records or instructions that requires verification rather than a guess about which version applies.

3. treatment hold

Turn 10. A treatment hold indicates an interruption under the actual plan and does not itself specify when or whether treatment should resume.

4. result ownership

Turn 14. Result ownership identifies who must review and act on a pending result so it is not lost between inpatient and outpatient services.

5. reconciled plan

Turn 18. A reconciled plan aligns verified instructions and responsibilities after discrepancies are resolved rather than combining conflicting records without review.

6. handoff acknowledgment

Turn 20. Handoff acknowledgment confirms that the receiving team obtained the information and accepted the relevant responsibilities rather than merely receiving a file.

Return to the conversation, page 98

Transfer exchange

1. pending

Hospital clinician: The result is still pending.

Turn 1. The result is not yet available in the supplied update.

Additional case 2 / Explanations / continued

2. named

Oncologist: The reviewing clinician was not named.

Turn 2. The summary omitted who was responsible for reviewing the result.

3. ownership

Hospital clinician: We need explicit ownership.

Turn 3. The teams must assign responsibility rather than assume another service will act.

4. result

Oncologist: No conclusion can be drawn from an unavailable result.

Turn 4. A pending result cannot support an interpretation before it exists.

Return to the conversation, page 101

Additional case 3 / Explanations

Extended conversation

1. cost-related barrier

Turn 4. A cost-related barrier is a practical difficulty affecting access and is not equivalent to an informed refusal of treatment.

2. out-of-pocket costs

Turn 6. Out-of-pocket costs are amounts the patient may need to pay under the actual coverage arrangements and cannot be guaranteed from an unverified estimate.

3. patient navigation

Turn 8. Patient navigation helps coordinate practical access and support across services without replacing the clinical plan or guaranteeing eligibility.

4. Eligibility review

Turn 12. Eligibility review checks whether the program's actual criteria are met and is separate from submitting an application or receiving approval.

5. treatment tradeoff

Turn 16. A treatment tradeoff concerns the benefits, harms, and practical implications of actual options and requires an appropriate clinical discussion rather than an automatic cost-based substitution.

6. Financial toxicity

Turn 20. Financial toxicity describes the harmful financial burden associated with care and deserves attention without blaming the patient or promising unverified assistance.

Return to the conversation, page 103

Transfer exchange

1. submitted

Patient: The application has been submitted.

Turn 1. The support team has submitted the application but has no decision yet.

Additional case 3 / Explanations / continued

2. unconfirmed

Navigator: Eligibility remains unconfirmed.

Turn 2. The program has not yet established eligibility or approved coverage.

3. authorized

Patient: Contact with the program is authorized.

Turn 3. The patient has permitted contact with the assistance program.

4. employer

Navigator: That does not authorize contacting your employer.

Turn 4. The supplied permission does not extend to the patient's employer.

Return to the conversation, page 105

Find a phrase by purpose

Explaining a new cancer diagnosis

Prepare for the finding: I am afraid the result is not what we hoped for.

Name the diagnosis: The tissue examination confirms a malignant tumor.

Allow a pause: We can stop here for a moment.

Full phrase bank, page 7

Separating stage, grade, and prognosis

Introduce the contrast: These two terms answer different questions.

Explain grade: Grade describes microscopic features of the tumor.

Explain stage: Stage describes the extent using the system for this cancer.

Full phrase bank, page 17

Discussing biomarker results

Acknowledge hope: I understand why a possible match feels important.

State the finding: The report identifies a tumor alteration that may be relevant.

Separate two steps: Finding a target and choosing a treatment are different steps.

Full phrase bank, page 29

Making treatment intent explicit

State the aim: The aim of this option is to control the disease.

Avoid a guarantee: An intended benefit is not a promised outcome.

Ask about daily life: How would the treatment demands affect caring for your partner?

Full phrase bank, page 39

Find a phrase by purpose / continued

Escalating a treatment-related symptom report

Lead with urgency: I am calling about an urgent symptom report after recent treatment.

State the action: The urgent assessment pathway is already activated.

Attribute the report: Lee reports fever and shaking.

Full phrase bank, page 51

Discussing a response-assessment scan

State the observation: This measured lesion is smaller than on the comparison scan.

Recognize its value: That is a useful finding to discuss.

Separate the review: The complete response assessment is still under review.

Full phrase bank, page 62

Explaining a clinical trial invitation

Introduce research: This is an option to discuss, not an obligation.

State uncertainty: The study is asking a question that is not already settled.

Explain assignment: Random assignment determines the study group.

Full phrase bank, page 74

Discussing palliative care and priorities

Ask what was heard: What did the phrase palliative care suggest to you?

Correct the inference: This referral is not a decision to stop treatment.

Explain the purpose: The team can help with symptoms and distress alongside other care.

Full phrase bank, page 84

Vocabulary index

acquired resistance
Lesson 03, p. 28

actionability
Lesson 03, p. 27

adjuvant treatment
Lesson 04, p. 37

advance care planning
Lesson 08, p. 82

advance directive
Lesson 08, p. 82

adverse event
Lesson 05, p. 49

alternative option
Lesson 04, p. 38

Appointment confirmation
Case 1, p. 93

assessment criteria
Lesson 06, p. 60

benefit-risk balance
Lesson 04, p. 38

biomarker
Lesson 03, p. 27

biopsy
Lesson 01, p. 5

cancer stage
Lesson 02, p. 15

care navigator
Lesson 01, p. 6

care preference
Lesson 08, p. 82

caregiver strain
Lesson 08, p. 83

caregiving responsibility
Lesson 04, p. 38

causality assessment
Lesson 05, p. 49

clinical handoff
Lesson 01, p. 6, Lesson 02, p. 16, Lesson 03, p. 28, Lesson 04, p. 38, Lesson 05, p. 50, Lesson 06, p. 60, Lesson 07, p. 73, Lesson 08, p. 83

clinical response
Lesson 06, p. 59

clinical stage
Lesson 02, p. 15

clinical trial
Lesson 07, p. 72

comfort-focused care
Lesson 08, p. 82

companion diagnostic
Lesson 03, p. 27

comparison scan
Lesson 06, p. 59

complete response
Lesson 06, p. 59

concurrent care
Lesson 08, p. 82

consent discussion
Lesson 07, p. 73

control group
Lesson 07, p. 72

cost-related barrier
Case 3, p. 102

curative intent
Lesson 04, p. 37

decision capacity
Lesson 04, p. 38

dehydration
Lesson 05, p. 49

diagnostic certainty
Lesson 01, p. 6

diagnostic disclosure
Lesson 01, p. 5

differentiation
Lesson 02, p. 15

discharge summary
Case 2, p. 97

disease control
Lesson 04, p. 37

distant metastasis
Lesson 02, p. 15

documented plan
Lesson 01, p. 6, Lesson 02, p. 16, Lesson 03, p. 28, Lesson 04, p. 38, Lesson 05, p. 50, Lesson 06, p. 61, Lesson 07, p. 73, Lesson 08, p. 83

eligibility criteria
Lesson 07, p. 72

Eligibility review
Case 3, p. 102

Vocabulary index / continued

emotional acknowledgment
Lesson 01, p. 5

escalation acknowledgment
Lesson 05, p. 50

extravasation
Lesson 05, p. 49

febrile episode
Lesson 05, p. 49

follow-up consultation
Lesson 01, p. 6

germline variant
Lesson 03, p. 27

handoff acknowledgment
Case 2, p. 97

hospice care
Lesson 08, p. 82

imaging studies
Case 1, p. 93

indeterminate finding
Lesson 02, p. 16

information overload
Lesson 01, p. 6

informed consent
Lesson 04, p. 38

interdisciplinary support
Lesson 08, p. 83

last treatment date
Lesson 05, p. 50

local treatment
Lesson 04, p. 37

malignancy
Lesson 01, p. 5

measurement variability
Lesson 06, p. 60

metastatic disease
Lesson 02, p. 16

mucositis
Lesson 05, p. 49

neoadjuvant treatment
Lesson 04, p. 37

neutrophil count
Lesson 05, p. 49

nodal involvement
Lesson 02, p. 16

enrollment
Lesson 07, p. 72

evidence level
Lesson 03, p. 27

family meeting
Lesson 08, p. 83

Financial toxicity
Case 3, p. 102

genetic counseling
Lesson 03, p. 28

goals-of-care discussion
Lesson 08, p. 82

histology
Lesson 01, p. 5

imaging finding
Lesson 02, p. 15

immunosuppression
Lesson 05, p. 49

infection concern
Lesson 05, p. 49

information preference
Lesson 01, p. 5

infusion reaction
Lesson 05, p. 49

investigational treatment
Lesson 07, p. 72

local extent
Lesson 02, p. 15

localized disease
Lesson 02, p. 16

measurable lesion
Lesson 06, p. 59

medication discrepancy
Case 2, p. 97

molecular alteration
Lesson 03, p. 27

multidisciplinary team
Lesson 01, p. 5

neutropenia
Lesson 05, p. 49

new lesion
Lesson 06, p. 59

non-target lesion
Lesson 06, p. 59

Vocabulary index / continued

out-of-pocket costs
Case 3, p. 102

overall survival
Lesson 07, p. 72

palliative care
Lesson 08, p. 82

partial response
Lesson 06, p. 59

pathological stage
Lesson 02, p. 15

pathology report
Lesson 01, p. 5, Case 1, p. 93

patient navigation
Case 3, p. 102

patient preference
Lesson 01, p. 6, Lesson 02, p. 16, Lesson 03, p. 28, Lesson 04, p. 38, Lesson 05, p. 50, Lesson 06, p. 60, Lesson 07, p. 73, Lesson 08, p. 83

performance status
Lesson 04, p. 37

predictive biomarker
Lesson 03, p. 28

primary endpoint
Lesson 07, p. 72

primary tumor
Lesson 01, p. 5

prognostic factor
Lesson 02, p. 16

prognostic uncertainty
Lesson 01, p. 5

progression-free survival
Lesson 07, p. 73

progressive disease
Lesson 06, p. 59

qualified interpreter
Lesson 01, p. 6, Lesson 02, p. 16, Lesson 03, p. 28, Lesson 04, p. 38, Lesson 05, p. 50, Lesson 06, p. 60, Lesson 07, p. 73, Lesson 08, p. 83

quality of life
Lesson 04, p. 37

radiological response
Lesson 06, p. 59

randomization
Lesson 07, p. 72

read-back
Lesson 05, p. 50

receiving clinician
Lesson 05, p. 50

reconciled plan
Case 2, p. 97

record-transfer authorization
Case 1, p. 93

recurrence
Lesson 06, p. 60

referral packet
Case 1, p. 93

regional lymph node
Lesson 02, p. 15

remission
Lesson 06, p. 60

research coordinator
Lesson 07, p. 73

research protocol
Lesson 07, p. 72

response assessment
Lesson 06, p. 59

restaging
Lesson 06, p. 60

result ownership
Case 2, p. 97

resuscitation status
Lesson 08, p. 82

scan interval
Lesson 06, p. 60

screening visit
Lesson 07, p. 72

second opinion
Lesson 01, p. 6

sequencing panel
Lesson 03, p. 27

serious-illness conversation
Lesson 08, p. 82

somatic variant
Lesson 03, p. 27

Vocabulary index / continued

source attribution
Lesson 01, p. 6, Lesson 02, p. 16, Lesson 03, p. 28, Lesson 04, p. 38, Lesson 05, p. 50, Lesson 06, p. 61, Lesson 07, p. 73, Lesson 08, p. 83

specimen adequacy
Lesson 03, p. 27

spiritual concern
Lesson 08, p. 83

stable disease
Lesson 06, p. 59

stage group
Lesson 02, p. 15

staging completion
Lesson 02, p. 16

staging investigation
Lesson 02, p. 15

staging workup
Lesson 01, p. 5

standard of care
Lesson 07, p. 72

study arm
Lesson 07, p. 72

support person
Lesson 01, p. 6

supportive treatment
Lesson 04, p. 38

surrogate decision-maker
Lesson 08, p. 82

symptom burden
Lesson 08, p. 83

symptom management
Lesson 08, p. 82

symptom onset time
Lesson 05, p. 50

systemic therapy
Lesson 04, p. 37

target lesion
Lesson 06, p. 59

targeted therapy
Lesson 03, p. 27

teach-back
Lesson 01, p. 6, Lesson 02, p. 16, Lesson 03, p. 28, Lesson 04, p. 38, Lesson 05, p. 50, Lesson 06, p. 60, Lesson 07, p. 73, Lesson 08, p. 83

testing limitation
Lesson 03, p. 28

therapeutic misconception
Lesson 07, p. 73

TNM classification
Lesson 02, p. 15

treatment access
Lesson 03, p. 28

treatment burden
Lesson 04, p. 37

treatment cycle
Lesson 04, p. 37

treatment hold
Case 2, p. 97

treatment intent
Lesson 04, p. 37

treatment regimen
Lesson 04, p. 37

treatment summary
Case 1, p. 93

treatment tradeoff
Case 3, p. 102

treatment-related toxicity
Lesson 05, p. 49

tumor grade
Lesson 02, p. 15

tumor heterogeneity
Lesson 03, p. 28

tumor profiling
Lesson 03, p. 27

urgent assessment
Lesson 05, p. 50

values clarification
Lesson 08, p. 83

variant of uncertain significance
Lesson 03, p. 27

voluntary participation
Lesson 07, p. 73

warning statement
Lesson 01, p. 5

withdrawal
Lesson 07, p. 73

References and publication notes

All cases, organizations, figures, and conversations are fictional teaching material, not transcripts. Communication practice does not replace qualified advice or current workplace procedures.

[1] National Cancer Institute. Cancer Staging.

Terminology context for extent of disease and staging; no fictional case supplies a staging algorithm. Source checked 10 October 2026.

https://www.cancer.gov/about-cancer/diagnosis-staging/staging

[2] National Cancer Institute. Biomarker Testing for Cancer Treatment.

Background on biomarker findings, treatment relevance, and the limits of a possible match. Source checked 10 October 2026.

https://www.cancer.gov/about-cancer/treatment/types/biomarker-testing-cancer-treatment

[3] National Cancer Institute. Palliative Care in Cancer.

Context for supportive care alongside treatment and the distinction from hospice care. Source checked 10 October 2026.

https://www.cancer.gov/about-cancer/advanced-cancer/care-choices/palliative-care-fact-sheet

[4] National Cancer Institute. Taking Part in Cancer Treatment Research Studies.

Background for voluntary research participation and informed-consent discussions. Source checked 10 October 2026.

https://www.cancer.gov/publications/patient-education/crs.pdf

Companion course

The website offers the eight-lesson course and related practice. This book can be used independently. No audio access or external account is needed for its exercises.

https://englishladder.com/efsp-oncologists.html

Explain precisely. Leave room to respond.

Eight oncology encounters practice difficult news, staging language, biomarker uncertainty, treatment choices, symptom escalation, response assessment, trial discussions, and care goals.

Explaining a new cancer diagnosis

The diagnosis is established in this case, but stage, treatment, and prognosis are not. Do not soften a confirmed finding into vagueness or turn it into an unsupported prediction.

Separating stage, grade, and prognosis

Grade and stage are not synonyms. Staging systems differ by cancer type; do not assign a TNM category or stage from a vocabulary definition.

Discussing biomarker results

A molecular alteration does not guarantee a useful treatment match or response. A tumor finding is not automatically an inherited finding.

Making treatment intent explicit

Curative intent states an aim, not a guarantee. Disease control and symptom relief are also meaningful but different goals.

Phil Stilwell

© 2026 Phil Stilwell. All rights reserved.

No part of this publication may be reproduced, stored in a retrieval system, or transmitted in any form or by any means without prior written permission from the author, except as permitted by law.

English Ladder / English for Work / Revised 2026 edition

This publication includes AI-generated text and cover illustration, revised and checked during production. Brand artwork remains the English Ladder mark. This edition makes no claim of independent professional certification.

Typeface: Bitstream Vera, embedded under its accompanying license. Illustration created with OpenAI's native image generator for this edition. The copyright notice is not a claim to ownership of third-party frameworks or references.